Competence, Assessment, and Diagnosis

7

LEARNING OBJECTIVES

7.1 Explain the processes by which counselors develop, maintain, and increase their competence to practice.

7.2 Describe the major ethical and legal issues involved in evaluation and assessment.

7.3 Discuss the unique ethical and legal challenges that counselors encounter when they diagnose mental and emotional disorders.

FOCUS QUESTIONS

1. If you were a prospective client seeking counseling, how would you go about finding a counselor who is competent to help you?
2. How do counselors determine whether they are competent to counsel a particular client?
3. Do you think it is appropriate for a counselor to refer a client whose beliefs and behaviors conflict with the personal values of the counselor?
4. What do you think should be done if a counselor becomes overwhelmed by personal problems and is not counseling effectively?
5. Evaluation as an end in itself is a very different process from counseling. Why would a counselor want to be an evaluator?
6. To what degree do you believe counselors are competent to administer and interpret psychological tests?
7. The *DSM-5* system for diagnosis is based on pathology and the medical model of mental illness, whereas counseling is grounded in the wellness model. Why must counselors learn to diagnose clinical disorders?

In this chapter, we explore three important areas of counselor practice: counselor competence, assessment and testing by counselors, and the diagnosis of mental disorders of clients by counselors. Counselor competence is difficult to assess because there are no absolute means to determine whether a counselor is competent, yet counselors are required to develop and maintain their competence to provide mental health services to clients. Counselors engage in assessment and testing in their practices, yet state statutes and the extent of counselor training limit the degree to which counselors use testing in their practices. To gain employment and have their services reimbursed by health insurance companies, counselors must diagnose mental disorders of their clients even though the philosophy of counselors embraces wellness and rejects the medical model on which diagnosis is based.

The Impact of the COVID-19 Pandemic on Counselor Competence, Assessment, and Diagnosis

A myriad of challenges to counselor competence arose in the immediate aftermath of the COVID-19 lockdown. Counselors needed to develop new competencies to provide online counseling services, including mastery of the technologies required to deliver services in a secure manner. Burnout—an

erosion of competence—became an increasing concern due to the stressors the pandemic placed on counselors' personal and professional lives.

During the pandemic lockdown, counselors transitioned to counseling their clients virtually. Mental health counselors needed to continue to assess their clients and provide diagnoses, but they were forced to do so in the absence of many of the visual, auditory, olfactory, and spatial cues on which they relied.

In addition, counselors have changed their approaches to assessment and testing due to the virtual counseling they are now practicing as a result of the COVID-19 pandemic. Counselors often assess the needs of their clients in initial interviews, which used to be conducted in face-to-face office settings. Subtle body language information is more difficult to assess when virtual counseling takes place. In addition, some psychological tests that counselors administer and interpret in their practices have had to be adapted to counseling that takes place at a distance.

The process counselors use to diagnose clients has not changed dramatically with the shift from in-person to virtual counseling. Counselors in private practices and in many other settings still must use the *DSM-5-TR* (American Psychiatric Association [APA], 2022) to render professional diagnoses. The diagnosis process can be accomplished in virtual counseling sessions. Technology is being used to create innovative approaches to teaching diagnosis skills to counseling students. For example, students have role-played interviews with client avatars in an online, 3D, virtual counselor training facility and perceived this to be an effective method to develop mental health interviewing and diagnosis skills (Lowell & Alshammari, 2019).

A new technology that has exploded onto the scene in counseling is artificial intelligence (AI). While the potential of artificial intelligence is seemingly endless, its rapid growth raises a host of ethical concerns. Artificial intelligence is deceptively simple to use, and mental health counselors who use psychological artificial intelligence or recommend it to clients with inadequate knowledge and training run the risk of practicing beyond their boundaries of competence (Fulmer et al., 2021). Artificial intelligence shows great promise for improving counseling services, but further research is needed to address the ethical and societal concerns that are raised by its use (Fiske et al., 2019). Although there is abundant room for caution, some evidence is appearing that artificial intelligence can serve as a cost-effective and accessible supplement to mental health counseling. For example, Fulmer et al. (2018), in a randomized controlled test, demonstrated that the use of a particular psychological artificial intelligence test (Tess) produced a significant decrease in depression and anxiety among college students. As of mid-2023, researchers were experimenting with using AI to collect diagnostic information from clients interactively (Roy et al., 2023) and with using ChatGPT as a diagnostic tool (Yang et al., 2023). AI may prove to be a valuable tool for counselors to use in treatment, diagnosis, and assessment. However, as was the case when older technologies were initially integrated into counseling, the innovations are proceeding at a more rapid pace than ethical concerns can be addressed.

COMPETENCE AS AN ETHICAL AND LEGAL CONCEPT

When clients come for counseling, they invest a great deal of reliance and trust in their counselors. The client's role in the therapeutic relationship—which involves dependency, self-disclosure, vulnerability, and expectations of finding relief and solutions to problems in a safe environment—underscores the counselor's obligation to provide competent services. When clients put their trust in counselors as professionals, one of their most fundamental expectations is that their counselors will be competent (Pope et al., 2021). Herlihy and Corey (2015a) noted that the trust that clients bestow on us is "a source of power that must not be abused; clients need to be able to rely on their counselor's competence as a helper" (p. 183).

Competence in counseling is not easy to define. According to Welfel (2016), competence involves a combination of *knowledge, skill*, and *diligence*. Knowledgeable counselors have a thorough grounding in the core areas of study required to practice the profession. As we noted in Chapter 2, the Council for Accreditation of Counseling and Related Educational Programs (CACREP, 2024) has identified core areas of instruction, including professional orientation and ethical practice, social and cultural diversity, human growth and development, career development, helping relationships, group work, assessment, and research and program evaluation. Most counselor training programs, whether or not they are CACREP-accredited, offer instruction in these basic areas. *Knowledge* is acquired through attaining a graduate degree in counseling and is maintained through continuing education activities.

Skill building is a complex process that begins in graduate school with applied courses such as counseling techniques, group work, and practicum and internship. Skills are then honed and expanded through post-degree, supervised counseling experience. Skilled counselors are able to select and use appropriately a range of basic counseling techniques and therapeutic interventions.

Diligence has been defined by Welfel (2016) as a consistent attentiveness to the client's needs that means putting client welfare first and foremost, above all other concerns. The first standard of the ACA *Code of Ethics* (2014) reads, "The primary responsibility of counselors is to respect the dignity and to promote the welfare of clients." Evidence of diligent practice includes being willing to do extra reading, research, training, consultation, and follow-up to ensure that clients are served effectively.

Counselors are neither totally competent nor totally incompetent, although legal arguments during malpractice lawsuits might imply otherwise. Competency is not an abstract concept; instead, it is based on performance. Neither is competency a dichotomous concept: Instead, there is a continuum of professional expertise, with gross negligence at one extreme and maximum effectiveness at the other extreme. As discussed in Chapter 1, the difference between law and ethics is that law demands a minimum level of practice from counselors, whereas ethics encourages counselors to aspire to the highest possible level of practice. Competency is a parallel concept in that external forces require counselors to demonstrate minimum competency for professionals, whereas an internal force demands that counselors strive for ideal practice. For example, state licensure boards set the *minimum* requirements that counselors must meet to practice in that state, and counseling graduate programs require *minimum* levels of performance for students to be granted a degree. On the other hand, counselors are constantly striving to attain *maximum* knowledge and skills, and national voluntary credentialing organizations such as the National Board for Certified Counselors (NBCC) certify counselors who have distinguished themselves by going beyond the minimum in the field.

So, as we discuss competence in this chapter, keep in mind that competency is not an either/or concept. Rather, competence is a complex construct that has many possible levels along a continuum.

Competency in counseling involves both ethical and legal considerations. From an ethical perspective, the most salient moral principle is *nonmaleficence*: Do no harm. Incompetence is often a major factor in causing harm to clients. Counselors rarely intend to harm their clients, but harm *can* occur if counselors are not knowledgeable, skillful, and capable.

The most basic ethical standard related to competence in the ACA *Code of Ethics* (2014) is that counselors "practice only within the boundaries of their competence." How can a counselor determine just where these boundaries lie? It is difficult to answer this question because counseling is an exceptionally broad profession. Just as attorneys and physicians could never be competent to practice in every area within law and medicine, counselors could never be competent to offer counseling services in all areas of practice or to everyone who seeks their services (Barnett & Johnson, 2015).

Competence is also a legal issue because society expects professionals to be competent and holds them to this standard through licensing boards and the courts. Legal issues relating to competence include state licensure board complaints and the law of malpractice. Neukrug et al. (2001) found counselor incompetence to be the second-most frequently reported area of ethical complaints

(after dual relationships with clients) in their survey of state counselor licensure boards, although in a later survey, Wilkinson et al. (2019) did not find incompetence to be a frequent cause for complaints. When a client is harmed, the counselor could be sued for malpractice and held legally responsible in a court of law. Many lawsuits brought by clients alleging that they were harmed focus on competence.

In this chapter, we first explore methods of developing and assessing competence to *enter* the counseling profession. Next, we focus on ways to *maintain* competence during one's professional career. Competence is a fluid concept, and achieving competence at one time does not ensure that one will remain competent over one's entire career (Barnett & Johnson, 2015). In addition, we address issues that arise when a counselor's competence is *questionable*. These issues include counselor burnout and counselor impairment.

Counselor Preparation

Competence is based on "education, training, supervised experience, state and national professional credentials, and appropriate professional experience" (ACA, 2014, §C.2.a.). To be competent, counselors are also expected to "gain knowledge, personal awareness, sensitivity, dispositions, and skills pertinent to being a culturally competent counselor in working with a diverse client population" (ACA, 2014). Counselor preparation is obviously a basic component in developing competence to counsel. The process of becoming competent to enter professional practice begins with university counselor preparation programs. Measuring a student's competency as a counselor has proven to be a difficult process because of the complexity involved in assessing the interrelated domains of knowledge, skills, and dispositions (Kimball et al., 2019). It has been suggested that students, particularly those enrolled in online counselor education programs, need to possess *digital competence* as well (Dixon-Saxon & Buckley, 2020).

The initial responsibility for producing competent practitioners lies with those who do the preparation: counselor educators and supervisors. During the period of time when prospective counselors are working toward their master's degrees and are working under supervision toward licensure, the responsibility for ensuring an acceptable level of competence rests with counselor educators and supervisors. These professionals have an ethical obligation to protect the public by acting as *gatekeepers* for the profession (Gilbert et al., 2019; Henderson, 2010; Kimball et al., 2019; McCaughan & Hill, 2014; Muse-Burke et al., 2018; Smarinsky et al., 2023; Swank & Smith-Adcock, 2014). When students are unable to demonstrate competence, there is potential for endangering or harming not only clients but also counselor educators, the reputation of counseling programs, and other students (Teixeira, 2017). Gatekeeping is discussed in detail in Chapter 14.

Counselor educators are keenly aware of their responsibility to select and admit to graduate programs the individuals who are likely to succeed at developing the skills, knowledge, and characteristics needed to become effective counselors. Admissions committees in CACREP-accredited programs must select applicants based on their "relevance of career goals, aptitude for graduate-level study, potential success in forming effective counseling relationships, and respect for cultural differences" (CACREP, 2024, p. 5).

Academic ability is one important factor. Selection criteria typically include grade point averages and scores on standardized tests such as the Graduate Record Examination (GRE) or Miller Analogies Test (MAT). Although some graduate programs in counseling have eliminated the GRE requirement in the belief that it discourages students of color from applying and discriminates against minority applicants, this was not a significant factor for the participants in a study by Hipolito-Delgado et al. (2021). These researchers cautioned against the continued use of a deficit perspective in admissions standards. Hammonds and Purgason (2022) have also called for a more holistic approach to admissions.

It is entirely possible for a student to have strong intellectual abilities and still not possess the personal characteristics needed to be a *therapeutic person*. Personal characteristics such as being open

to new ideas, having flexibility, being cooperative, being open to feedback, having an awareness of one's impact on others, having the abilities to deal with conflict and to express one's own feelings, being able to reason using higher levels of cognitive development, and accepting personal responsibility have been identified as necessary for success as a counselor (Lloyd-Hazlett et al., 2017; McAdams et al., 2007). In addition, having a tolerance for ambiguity and a willingness to explore one's own biases, values, and personal issues have been shown to be related to the ability to develop effective therapeutic relationships. Counselor educators regularly assess student dispositions, which are "a measure of functioning in the personal and professional realms composed of nine factors (cognitive, ethical/legal, interpersonal, personal wellness, personal–professional boundaries, professionalism, responsiveness, self-control, and suitability for the profession" (Miller et al., 2019, p. 11).

Although these characteristics are difficult to measure, most counselor preparation programs rely on a personal interview, written essay, or other subjective criteria in selecting candidates for admission. The admissions criteria most commonly used for master's-program admission among CACREP-accredited programs surveyed by Swank and Smith-Adcock (2014) were letters of recommendation (96%), GPA (95%), personal statement (89%), and interviews (82%).

Once candidates have been selected and admitted into a graduate program, the next question that arises is "What does a master's-degree program need to include in order to produce competent counselors?" As we described in Chapter 2, CACREP/CORE has developed standards for preparing counselors. CACREP/CORE accredits preparation programs that have undergone a rigorous review, so it can be reasonably assumed that graduates of approved programs possess certain competencies (Herlihy & Corey, 2015a). It should be noted, however, that many counselor preparation programs are not CACREP/CORE accredited, and many competent counselors are educated in programs of sound quality that have not sought this accreditation. Another approach to ensuring adequacy of preparation programs is regional accreditation of universities. Regional accrediting agencies have set the minimal standard for universities in the United States. If a university is accredited by a regional accrediting agency, its legitimacy is assumed. The ACA *Code of Ethics* (2014) requires that "counselors accurately represent the accreditation status of their degree program and college/university." Counselors should not explicitly state or imply that the university that granted their degree was regionally accredited if it was not accredited at the time that the degree was granted.

Although these approaches to ensuring quality of counselor preparation are both necessary and helpful, the reality remains that successful graduation from an accredited program does not always guarantee competence (Kitzrow, 2002). Much depends on the individual student's motivation and ability to learn, the quality of instruction, the competencies of the faculty members, the breadth and extent of the supervised counseling experience provided by the program, and the quality of supervision received. In our own experience as counselor educators, we have found that most students who are about to graduate are keenly aware that they have only just begun to develop competence as practitioners. As a novice counselor, you need not be hobbled by a concern over your limitations and lack of experience. If you intend to seek counselor licensure, you will continue to be supervised closely and supported in your work as a counselor until you have been endorsed for independent private practice. Nonetheless, a healthy awareness of your limitations can help you avoid exceeding your boundaries of competence in your eagerness to begin practicing your new profession (Binkley & Leibert, 2015).

Credentialing

As was discussed in Chapter 2, two important types of credentialing found in counseling are certification—which takes various forms—and licensure. These credentials provide a tangible indicator of certain accomplishments, and, as such, they have implications for assessing the

competence of the credential holder. Counselor credentials were discussed in Chapter 2 and are revisited in this chapter, with a focus on how credentials are related to determining counselor competence.

The terms *licensure, certification*, and *registration* have many different meanings but are sometimes used interchangeably, which causes a great deal of confusion. These terms were explained in depth in Chapter 2 and are briefly restated here. *Licensure* refers to the most rigorous form of regulation in that only those who are licensed may practice the profession in that state. *Certification* is the term used when a title, such as *professional counselor*, can be used only by those who are certified; but anyone can practice the profession without being certified. *Registration* is a form of governmental regulation in which members of a profession must *sign up* with the state government if they practice the profession in that state. However, anyone may sign the registry without a review of their credentials (Anderson & Swanson, 1994). National private organizations, such as the NBCC, offer national certification, which is a credential that is voluntary (i.e., not required by a government for practice). In addition, state departments of education require certification of school counselors employed in public school districts. In this chapter, we use the term *licensure* to refer to state regulation of a profession generally and the term *certification* to refer to national voluntary credentials and state department certification of school counselors.

LICENSURE In Chapter 2, we explained that all 50 states, the District of Columbia, and Puerto Rico now license counselors, but many exemptions to the licensure requirement exist. The differences in state laws that license, certify, or register counselors were explained in Chapter 2 as well.

Licensure is related to counselor competency in that it is the most powerful type of credentialing and is established by state law. When state legislators determine that the public's best interests are best served by creating a license to practice a particular profession, the minimum standards for practice of that profession in that state are established through a political process. Because state governments would rather not be involved in regulating the practice of professions, legislators agree to license a professional group only when it can be shown that members of the public (a) do not have the ability to determine who is competent to practice within a particular profession and (b) could be harmed by incompetent practitioners.

When a state legislature passes a statute that establishes counselor licensure or passes amendments to an existing licensure statute, part of that statute specifies the minimum standards for becoming licensed to practice in that state. When these bills or amendments are introduced, political pressures are put on politicians. For example, it took many years for counselors to achieve licensure, after working against an array of forces. Economists argued that licensing of professions is self-serving, restricts entry into professions, and causes fees for services to rise. Organizations whose purpose is to limit the role of government in citizens' lives also opposed licensure statutes. They often were joined in their opposition by other mental health professionals (including psychologists, psychiatrists, and social workers) who did not want to see counselors licensed to provide mental health services because marketplace competition would increase. When these groups of related mental health professionals realized that a bill or amendment was going to be passed over their objection, they often demanded very high standards for licensure to keep as few individuals as possible from being licensed as counselors. All these groups put political pressure on legislators to set the minimum standards for licensure very high.

On the other side were pressures to set the minimum standards very low. For instance, individuals who were already practicing counseling in a state when counselor licensure statutes were being considered or amended often lobbied for statutes that included very low standards so that they would not lose their status and would still qualify to be licensed.

As a result of these various political pressures, states now have differing standards. A majority of states require 60 educational credit hours, but some states require specific coursework, such as family counseling, human sexuality, and addictions. States also vary in the numbers of required post–master's-degree supervised hours (Elliott et al., 2019).

Although there is variation from state to state, all licensed counselors (except those who were grandparented in) have completed at least a master's degree, have had post-master's supervised experience, and have successfully completed an examination or other form of screening. Thus, when clients select licensed professionals as their counselors, they can be assured that their counselors have demonstrated certain knowledge, skills, and abilities to the satisfaction of those professionals who evaluated them. Nonetheless, as is true of other approaches, there are limitations to what licensing can accomplish in terms of ensuring competence. Some have questioned whether there is evidence that licensure actually ensures general competence, protects consumers, or promotes higher standards of practice (Pope & Vasquez, 2021). It is also true that the licenses of mental health professionals are sometimes revoked for incompetent practice.

Another limitation to licensure is that the possession of a license does not ensure that practitioners will competently *do* what their license permits them to do. The counselor license is a generic one, which leaves it up to licensed counselors, as individuals, to discriminate among professional titles and the professional *functions* they are competent to perform. In addition, whereas licensure boards can restrict or terminate the practice of incompetent practitioners against whom they have received complaints, they are not in the business of monitoring the practices of counselors about whom they have received no complaints.

Clearly, licensing of counselors is an attempt to ensure competent practice. Government regulation may have a positive influence on competency within the counseling profession, but it falls short of accomplishing its objective of ensuring competence. In reality, the standards set for licensure are often the result of political compromises rather than of standards set by the profession for minimum competency.

CERTIFICATION In addition to becoming licensed, counselors can offer evidence that supports their competence by becoming certified. The various types of counselor certification, the history of state agency counselor certification and national voluntary counselor certification, and the requirements for certification were summarized in Chapter 2. Some types of certification are mandatory for practicing in certain settings and occur at the state level. For instance, public school counselors must be certified by the states in which they practice. There are also national, voluntary certifications, such as those offered by the NBCC. National Certified Counselors (NCC), like licensed counselors, have received preparation in specific content areas and clinical instruction, have had supervised counseling experience, and have passed an examination.

SPECIALTIES In addition to generic certification as an NCC, NBCC offers three specialty certifications: Certified Clinical Mental Health Counselor (CCMHC) and National Certified School Counselor (NCSC), and Master Addictions Counselor (MAC). An affiliate of NBCC, the Center for Credentialing and Education (CCE), offers the Board Certified TeleMental Health (BC-TMH) credential. Specialty certifications are another means of establishing a counselor's competence to work with certain types of clients, in certain settings, or in specialized areas of counseling. A host of other, narrower specialty certifications are offered by various groups in such areas as hypnotherapy, biofeedback, or sex therapy. Specialty licensing has been a controversial issue in counseling: Some have argued that only practitioners who have specialized preparation should be allowed to practice that specialty. Others have argued against this, noting that other professionals

such as physicians and lawyers may practice any type of medicine or law. The only force that restrains counselors in limiting themselves to specialties is their own understanding that they must be competent in the professional services they render and that no one can be competent in all areas. Specialty *accreditation* of counselor education programs is another approach that suggests that only certain counselors are qualified to practice in specified areas. CACREP has taken this approach, accrediting CACREP (2024) master's-degree programs in addictions; career; clinical mental health; clinical rehabilitation counseling; college counseling and student affairs; marriage, couple, and family counseling; rehabilitation counseling; and school counseling and doctoral programs in counselor education and supervision.

There appears to be tension between proponents of two schools of thought regarding specialty preparation. Some counselors believe that establishing competence boundaries should be left up to individual professionals, who then would be accountable to licensure or certification boards if they exceeded those boundaries to the detriment of a client. Others believe that counselors should be prevented by licensure boards from practicing within specialty areas unless they have received specific preparation in the form of graduate courses or supervision. The issues of specialty preparation and specialty licensing and certification will likely be debated within the counseling profession for some time. Currently, however, the prevailing school of thought is that counselors who have proven their minimal preparation and are licensed can decide for themselves the boundaries of their competence and may practice in areas considered specialties without holding any specialty certifications. Nonetheless, counselors have an ethical responsibility to practice in specialty areas new to them *only after* they have obtained appropriate education, training, and supervised experience; they must also take steps to ensure their competence at their work while developing skills in new, specialty areas (ACA, 2014, *Code of Ethics*, §C.2.b.).

Maintaining Competence

Once counselors have completed their formal education and are licensed or certified to practice, the burden of determining competence shifts away from counselor educators and supervisors and onto the counselors themselves. Counselors are autonomous professionals, responsible for monitoring their own effectiveness. The *Code of Ethics* underscores this responsibility, stating that "counselors continually monitor their effectiveness as professionals and take steps to improve when necessary" (ACA, 2014, §C.2.d.). Even when counselors are employed in agencies or institutions, they are held individually accountable for practicing in an ethical and professional manner. One of the indicators of a profession that distinguishs it from a semi-profession or a non-profession is that its members practice autonomously. In return for this privilege of independence, professionals must limit themselves to practicing within the areas in which they are competent. Thus, professionals must individually determine the limits of their competence and practice accordingly.

CONTINUING EDUCATION Although competence can certainly be enhanced by the skillful application on a day-to-day basis of the knowledge gained in a formal preparation program, experience alone is no guarantee against errors. Considering the constant contributions that new research makes to knowledge in the field, as well as the significant, ongoing changes in the environment in which counseling is practiced, we doubt that a counselor could retain even a modicum of professional competence over a 30- or 40-year career without further education. Counselors must therefore keep up with new knowledge related to emerging issues; for instance, there is a growing body of literature on counseling clients who have been victims of sex trafficking (e.g., Browne-James et al., 2021; Burt, 2019; Interiano-Shiverdecker et al., 2023; Litam & Lam, 2021).

The ACA *Code of Ethics* (2014) states that counselors must recognize the need for continuing education to acquire and maintain awareness of current scientific and professional information, and

most counselor licensure and certification boards have established continuing education requirements for maintaining one's license. NBCC-approved continuing education workshops are widely available over the internet. Wilkinson et al. (2019) found failure to acquire the appropriate amount of continuing education to be one of the most frequent types of ethical violation leading to disciplinary action by state licensing boards. It is likely that state licensure boards are disciplining counselors who do not prove they have completed the minimum number of continuing education hours that the boards have set rather than refusing to renew these counselors' licenses. As Herlihy and Corey (2015a) have noted, however, there are limits to what continuing education requirements can accomplish: "It is difficult to monitor the quality of continuing education offerings or their relevance to a particular counselor's needs. The number of clock hours obtained may have little relation to how much the counselor has actually learned and integrated into practice" (p. 185). The focus should instead be on maintaining competence rather than on simply accumulating the continuing education credits required to maintain a license or certification.

Many other questions could be raised regarding what constitutes a legitimate effort to maintain competence. For instance, is attending a seminar commensurate with teaching one? Is writing an article for a professional journal an indicator of continuing competence? What is the relative value of reading books and scholarly articles about new techniques and theories? Can counselors who regularly but informally consult with colleagues about their cases be considered as diligent in working to maintain competence as counselors who attend formal workshops and seminars? There may be no way to objectively assess whether a counselor has maintained competence over time. External criteria such as continuing education credits earned are not sufficient to ensure continuing competence. More important are counselors' own efforts, as autonomous professionals operating at the aspirational level of ethics, to learn about new developments in the field, remain aware of their own limitations, recognize that these limitations can increase over time, and seek to keep skills current by both formal and informal means.

7-1 The Case of Gretchen

Gretchen is a licensed professional counselor (LPC) in a community mental health agency. Gretchen received her license 6 months ago. For 3 years while she was working toward getting her license, she worked as a counselor intern at the agency. During her time as an intern, Juanito (Gretchen's administrative supervisor) was also Gretchen's clinical supervisor. Juanito expected Gretchen to attend the in-service trainings at the agency as well as professional conferences and workshops, and Gretchen complied. Now that she is licensed, Gretchen is irritated that the licensing board requires her to earn a certain number of continuing education hours every year. Gretchen believes that because she has recently completed 3 years of supervision that included a great deal of in-service training, her knowledge and skills are current. Realizing, however, that she must accrue continuing education hours to maintain her license, Gretchen attends a state counseling conference. She attends sessions, sits at the back of the room, and surreptitiously plays games on her cell phone. Gretchen enrolls in two webinars that she can attend online and barely listens to the presenters as they cook meals and do household chores.

- Do you believe Gretchen is correct in believing that her skills and knowledge are current and therefore she should not be required to accrue continuing education credits during her first year as a licensed counselor?
- Are Gretchen's methods of accruing their required post-licensure continuing education credits ethical?

Discussion: Gretchen may be correct that her skills and knowledge are current; however, her behaviors seem to suggest that she believes that she does not need to continue learning and developing as a counselor. Gretchen is misunderstanding the purpose of continuing education, which is not only to keep current but also to improve one's performance as a counselor, rather than simply accruing a required number of credits. Although her strategies for accruing hours do not rise to a level that justifies a formal charge of unethical behavior, Gretchen's approach to continuing education is far from best practice and certainly could be considered unprofessional.

PEER REVIEW/PEER CONSULTATION Peer review, a system by which mental health professionals assess one another's services, is a convenient and cost-effective approach to monitoring competence (Olson et al., 2016). Peer review is useful for counselors at all levels of experience and should continue as a life-long learning process. The many benefits include enhanced self-awareness; opportunities to discuss a wide variety of answers to clinical and professional questions (Barnett & Johnson, 2015); mutual support; objective feedback in dealing with countertransference issues; information on therapeutic techniques, new research, and referral sources; and help in dealing with difficult cases, stress, and the isolation often experienced by private practitioners. Ideally, peer consultation or supervision groups would meet on a regular, scheduled basis to discuss practice challenges and ethical issues. Peer consultation has been recommended as a strategy for burnout prevention among school counselors (Brott et al., 2021), who may routinely receive administrative supervision but not clinical supervision. If a peer consultation group is not available, Johnson et al. (2012) recommended that counselors maintain close collegial relationships with several fellow professionals who can provide consistent and honest feedback. Peer supervision and consultation, along with opportunities to confer with an expert supervisor, have also been found to be helpful in learning new counseling approaches (ten Napel-Schutz et al., 2017). Independent private practitioners are expected to seek peer supervision (ACA, 2014, *Code of Ethics*, §C.2.d.), and this is a strategy we recommend for counselors in all work settings.

TECHNOLOGY The internet presents counselors with myriad opportunities for keeping current and improving their competence as practitioners. New information technologies are developing at such a rapid pace that any attempt we might make to describe them would be obsolete almost immediately. We can, however, highlight a few key resources.

The counseling profession has established a solid presence on the internet. This offers opportunities to access virtual libraries for researching the latest information on client problems and effective counseling techniques, as well as to collaborate and consult with other professionals around the world. Many of ACA's divisions, NBCC, and CACREP have websites so that you can communicate quickly and directly with professional groups that are working to strengthen counseling as a profession. Videoconferencing enables you and counselors everywhere to obtain further preparation and even supervision without having to travel great distances. You can also subscribe to mailing lists that allow you to share experiences, ask questions, and exchange information and ideas.

MAKING REFERRALS The ACA *Code of Ethics* (2014) states that counselors should avoid entering into a therapeutic relationship when they lack the competence to counsel a prospective client. If a counseling relationship has already begun, counselors should discontinue working with the client and suggest alternative resources that are culturally and clinically appropriate (ACA, 2014). Ethical counselors recognize that they will need to refer a client when accepting or continuing to work with

that client would exceed their boundaries of competence. Sometimes it can be difficult for counselors to acknowledge that they are not competent to provide professional services to every client who might request their help because there are many sources of temptation to accept clients who might be better served by a referral. These temptations might include a reluctance to disappoint a valued source of referrals, financial pressures to increase your client load when business has been slow, or the ego-enhancing nature of requests from clients who hold exaggerated beliefs about your abilities to help them. To succumb to these kinds of temptations would not be ethical and would not be in your clients' best interest.

An issue that has become the focus of much controversy is the distinction between competence-based referrals and values-based referrals. In Chapter 3, we discussed four lawsuits that involved counseling practitioners and counseling students who refused, based on their religious beliefs, to counsel LGBTGEQIAP+ clients regarding their same-sex relationships. These cases brought the issue of values-based referrals into bold relief. A standard was added to the ACA *Code of Ethics* (2014) that clarifies that counselors should not refer prospective or current clients "based solely on the counselor's personally held values, attitudes, beliefs, and behaviors." It is important to understand that referral of a prospective or ongoing client should be based on skill-based competence; a claim of lack of competence cannot be used as a justification for engaging in discrimination (Kaplan et al., 2017). Standard C.5 of the ACA *Code of Ethics* (2014) makes it clear that counselors cannot discriminate against an entire class of people based on the counselor's personal values (Bayne et al., 2021). As noted earlier, an example of this kind of inappropriate referral is referring all prospective LGBTGEQIAP+ clients on the basis of the counselor's religious beliefs. This type of inappropriate referral is not limited to conflicts related to sexual, affectional, intersex, and gender expansive identities and religious values. For instance, because counselors are expected to be competent to work with a diverse client population (ACA, 2014) and because it would be unethical to refuse to counsel an entire class of people (ACA, 2014), it would also not be acceptable for a counselor to refuse to accept clients who are older adults by claiming lack of competence in gerontological counseling. Instead, the counselor is expected to gain competence in working with older adults.

It can be difficult to distinguish referrals based on competence from those that are discriminatory. To clarify the distinction, Kaplan et al. (2017) offered the example of a client who decides to explore the possibility of gender reassignment. Assuming that no legal restrictions exist in the state that would affect this exploration, the counselor must still proceed with caution. The issue of gender reassignment surgery within a transgender developmental process is quite specialized. Thus, a counselor with no experience with gender reassignment surgery would be justified in suggesting a referral to another provider with experience and expertise. However, that referral would need to be based on lack of experience and lack of competence, not on the counselor's values and beliefs regarding gender reassignment. The issue of appropriate versus inappropriate referrals has been widely discussed and apparently is being covered in counselor training programs. For example, counseling students studied by Lloyd-Hazlett et al. (2017) demonstrated nuanced understandings of competence, values, and referral processes.

A particular type of challenge is posed when the issue of competence arises with respect to a client with whom a counselor is already engaged in a counseling relationship. Despite the best efforts of counselors to accept only those clients whom they believe they can provide with competent services, the course of counseling is unpredictable. A client's presenting problem may be well within the counselor's scope of competence, and the counselor might in good faith begin working with that client, only to discover as therapy progresses that the client has unanticipated counseling needs.

7-2 The Case of Jing

Jing, a licensed professional counselor (LPC) in private practice, began working with Ellen, a young woman who came for counseling to deal with what at first appeared to be moderate anxiety. Ellen described herself as a perfectionist and sought counseling to learn to "stop being so hard on herself." It was only after several counseling sessions that Ellen felt safe enough in the counseling relationship to reveal to Jing that she was so fearful of gaining "even an ounce" that she regularly engaged in self-induced vomiting after meals, abused laxatives, and exercised excessively. Jing recognized these behaviors as symptoms of anorexia nervosa or bulimia nervosa (APA, 2022), a problem area with which Jing had no experience.

- How should Jing approach this situation with Ellen?
- If Jing decides to take workshops and do independent reading on treatment of individuals who have eating disorders, how will Jing know when she is competent to provide counseling services to clients like Ellen?
- What if Jing is able to find a suitable referral source but Ellen refuses, stating that she has complete faith in Jing's ability to help her?
- What if Jing practices in an isolated, rural community and a local referral resource or expert consultant or supervisor is not available?

Discussion: Jing has an ethical obligation either to refer Ellen to a specialist or to another therapist who is competent to work with clients who suffer from eating disorders or to ensure that they provide quality counseling services to Ellen by obtaining educational experiences and consultation or supervision from experts. Also, because both anorexia nervosa and bulimia nervosa can be life-threatening conditions, Jing will need to develop a relationship with a local physician who will treat Ellen's physical issues at the same time.

If Jing decides to refer, it might be difficult for her, after establishing rapport with Ellen, to tell Ellen that she now needs to see a different counselor. Jing will have to be clear in explaining her reasons for making the referral and, at the same time, be sensitive to Ellen's concerns.

If Jing decides to take workshops and do independent reading on the treatment of individuals who have eating disorders, she will need to consider a number of issues in determining whether she is competent to provide counseling services to clients like Ellen. There are no rules about how a counselor adds a new area of expertise—Jing will have to decide for herself when she is competent in this new area. Should someone later challenge her regarding this competence, Jing should be ready to defend her new competence by providing evidence of how she developed it.

If Jing is able to find a suitable referral source, but Ellen refuses, Jing will need to be firm in refusing to continue to work with Ellen. If Jing has trouble being firm in a situation like this, she should seek clinical supervision for support in doing what has to be done.

If Jing practices in an isolated, rural community and no local referral resource, expert consultant, or supervisor is available, then there really is no absolutely correct way to proceed in a situation like this.

Jing might consider her options—for example, she could (a) enter into an internet supervisory arrangement with a specialist in eating disorders and continue to counsel Ellen under that person's clinical supervision and in collaboration with a physician who is monitoring Ellen's physical health or (b) investigate and find the closest and best treatment option available for Ellen and inform Ellen that this is really her best option, despite any travel hardships.

Other situations that might raise the question of referring a client are less clear-cut. Even the most experienced counselors will at times seriously wonder whether they have the personal and professional abilities to work with some of their clients. Difficulty in working with some clients does not, in itself, imply incompetence. In fact, counselors who refer all clients with whom they encounter difficulties will probably have few clients. Instead, counselors can extend their boundaries of competence through reading, participating in workshops and other professional development opportunities, consulting, co-counseling with colleagues who have expertise in a needed area, and working under supervision. The key is to keep a careful balance of referring when necessary and in other situations extending your areas of competency while keeping your clients. Counselors can prevent stagnation and can continue to learn and grow as professionals by taking on clients who present new concerns and issues, thus extending their scope of competence. In fact, it would be impossible to develop expertise in a new counseling specialty area without eventually accepting a client with an issue in that area. At the same time, counselors must take care to ensure that clients are not harmed while the counselor is in the process of learning new skills and developing new competencies. You must make careful judgments regarding when to refer and when to keep new clients as you are preparing to provide services in a new area of counseling.

Diversity Considerations

As we emphasized in Chapter 3, recognizing diversity in U.S. society and developing intercultural counseling competence are essential to ethical practice (Byrd & Hays, 2012; Schmidt et al., 2011). When you think of your own future practice, keep in mind that if you are not trained and competent to work with culturally diverse clients, you are at risk of practicing unethically if you attempt to provide services to such clients. Counselors have an ethical obligation to actively attempt to understand the diverse cultural backgrounds of their clients and to learn how their own cultural, racial, and ethnic identities have an impact on their values and beliefs about the counseling process. The *Code of Ethics* has incorporated, throughout the document, provisions requiring counselors to be culturally competent. The introduction to Section A (ACA, 2014) states, "Counselors [should] actively attempt to understand the diverse cultural backgrounds of the clients they serve. Counselors [should] also explore their own cultural identities and how these affect their values and beliefs about the counseling process." Every section of the *Code* reminds counselors of their duty to be aware of cultural differences in their counseling practices. Fortunately, the counseling profession's attention to cultural diversity has increased dramatically. Research on multicultural competence continues to appear at a rapid pace, and there is an abundance of up-to-date professional literature that can enhance counselor awareness and knowledge. Research results have provided abundant support for the importance of multicultural competence to the process and outcome of counseling (Barden et al., 2017; Dillon et al., 2016; Ivers et al., 2016; Johnson & Williams, 2015; Matthews et al., 2018; Oh et al., 2022; Swan & Schottelkorb, 2015), particularly for Black, Indigenous, and People of Color (BIPOC) clients (Bathje et al., 2022).

Counselors who work in urban and suburban settings typically have numerous referral resources available to them. Counselors who practice in rural areas, however, do not enjoy this luxury and often practice as generalists out of sheer necessity (Welfel, 2016). The geographic location of their practice places extra demands on them to increase their knowledge and skills to be able to respond to the varied needs of their clientele. At the same time, they must practice diligence, weighing the risk of causing harm if they provide services to a client who presents problems or issues in which they are not adequately trained. Fortunately, maintaining competence is no longer as difficult as it was in the past, now that opportunities to increase knowledge and skills are readily available through distance consultation and supervision, interactive webinars, and online continuing education courses.

Erosion of Competence: Burnout and Impairment

The work of the counselor can be stressful and emotionally demanding. Counselors, as partners in the therapeutic journey, can be deeply affected by the pain of their clients (Corey et al., 2024). This stress can take its toll on practitioners. Ironically, the counseling relationship itself can be a source of stress. In other interpersonal relationships, such as friendships, there is give and take and a reciprocal meeting of needs. This balance does not exist in a counseling relationship. Counselors are experts in one-way caring (Skovholt, 2001) who make a *loan of the self* to the therapeutic relationship (Squyres, 1986), receive little in return, and sometimes doubt the effectiveness of counseling. Of counselors surveyed by Farber and Heifetz (1982), 74% saw lack of therapeutic success as the most stressful aspect of their work, and 55% felt depleted by the unreciprocated attentiveness, giving, and responsibility that the therapeutic relationship demands. Stress can lead to distress, which can, over time, lead to burnout and impairment (Gutierrez & Mullen, 2016). The terms *distressed, burned out*, and *impaired* have been used somewhat interchangeably in the counseling literature. However, these terms might be better viewed as ranging along a continuum from the least to the most serious in relation to their impact on competent professional performance.

Most counselors probably could be described as *distressed* at some time during their professional lives. Distressed counselors may experience irritability, anxious and depressed moods, somatic complaints, lowered self-esteem, and feelings of helplessness, but they are not necessarily impaired in their professional functioning. They know at some subjective level that something is wrong and that distress is usually a transitory and temporary condition.

When distress remains unalleviated, however, it can lead to *burnout*, which has been described as "a psychological syndrome emerging as a prolonged response to chronic interpersonal stressors on the job" (Maslach & Leiter, 2016, p. 103). Burned-out counselors, exhausted and depleted, have little energy left for their clients. Common indicators of burnout include a decline in counselors' self-care, strain in personal relationships, and damage to their overall emotional health (Bardhoshi et al., 2019; Cook et al., 2020; Maslach & Leiter, 2016). Cook et al. (2021) found that novice counselors were also likely to experience fatigue and tiredness and feel unfulfilled in their work. According to Zur (2011a), signs of burnout include emotional depletion, an unrelieved sense of worry, loss of perspective, helplessness, inefficiency, and an inability to leave clients' concerns at the office. Recent literature suggests that burnout may be a distinct concern for school counselors (Bardhoshi & Um, 2021; Evans Zalewski, 2022; Fye et al., 2020; Hamelin et al., 2022; Mullen et al., 2020). Burnout is not as much a state or condition as it is a process that, if not corrected, can lead to impairment.

Concerns about counselor burnout were heightened during the COVID-19 pandemic. Counselors were dealing with the same stressors as their clients were facing, including increased financial hardships, social isolation, and increased mental health needs (Litam & Balkin, 2021). Research studies appeared at a rapid rate as counselors sought to understand their own experiences of the phenomenon. Litam et al. (2021) found that perceived stress was positively related to burnout and that resilience was negatively related to burnout during the COVID-19 pandemic in a national sample of professional counselors. These researchers stressed the importance of engaging in self-care activities and cultivating resilience. Cook and Fye (2022) examined predictors of burnout among pre-licensed counselors and found that COVID-19 anxiety was the strongest predictor of burnout for their study participants.

Villares et al. (2022) compared school counselors' burnout levels during COVID-19 by categorizing their approaches as reactive, universal, consultative/collaborative, and comprehensive. They found that reactive school counselors, who spent most of their time on non-counselor duties, reported the highest levels of burnout. The researchers stressed the importance of school counselors defining their roles so that they can deliver their services during stressful times. Without a doubt, the

long-term impact of the pandemic on counselors, counseling students, and counselor educators and supervisors will continue to be a topic of study in the post-pandemic era.

RISK FACTORS FOR IMPAIRMENT Even in less stressful times, it would be rare for a counselor to never experience a frustrating week, a difficult client, an emotional overload, or occasional symptoms of burnout, yet counselors who are functioning well can put these experiences into perspective. *Impaired counselors*, by contrast, are unable to transcend periods of stress (Stadler, 2001). Impaired counselors are unable to perform their responsibilities appropriately, putting their clients at risk. Impairment is often associated with alcohol and other drug abuse and with the blurring of therapeutic boundaries that can lead to sexual exploitation of clients.

Why do some counselors become impaired whereas others manage to bounce back from periods of distress or burnout? Sometimes *environmental factors* can play a key role. A counselor may experience the death of a loved one, divorce or desertion, physical or sexual assault, the severe physical illness of a family member, or other personal loss or trauma. These events can unbalance anyone's emotional equilibrium. If the counselor takes time off from work, goes for counseling, or in some other overt way practices self-care and works toward a return to full functioning, this type of impairment is usually transitory and need not be a cause for ongoing concern about the counselor's competence.

Events of recent years such as the COVID-19 pandemic, human-created tragedies such as mass shootings, and natural disasters such as hurricanes and widespread flooding have created a high likelihood that most counselors will work with traumatized clients (Brown, 2018; Krennerich et al., 2021; Pow & Cashwell, 2017; Tarvydas et al., 2017). Even when counselors' lives are not directly affected, these events can put counselors at risk of experiencing *vicarious traumatization*, also known as compassion fatigue or secondary traumatic stress disorder (Gentry et al., 2018; Sanabria, 2018). Counselors who fail to recognize the warning signs of vicarious traumatization in themselves may become less skilled and less able to be emotionally present with clients who have been traumatized. Thus, counselors have an ethical obligation to be aware of this risk and engage regularly in self-care.

In other instances, *preexisting conditions* may put the counselor at risk. Some individuals enter the helping professions to work through their own unresolved problems. Personal difficulties that have led some counselors to enter the profession may be exacerbated by the practice of the profession. The practice of counseling can reactivate early experiences, open old wounds, and reawaken unresolved issues. Counselors with a history of addictions or a vulnerability to substance abuse seem to be predisposed to problems after entering the profession. Brown et al. (2022) found a positive correlation between adverse childhood experiences and burnout in clinical mental health counselors. Glickhauf-Hughes and Mehlman (1996) contended that certain personality characteristics make counselors more vulnerable to impairment. These characteristics include parentification (a willingness to take responsibility for others), perfectionism, imposter feelings and self-doubt, and audience sensitivity (strong awareness of others' feelings and responsiveness to them).

Stressful or traumatic events in a counselor's personal or social life and preexisting conditions are not the only factors that can contribute to counselor impairment. Certain kinds of *clients* and *client problems* can promote counselor distress (Stadler, 2001). Working with clients who pose a danger to themselves has been shown to take an emotional toll on counselors. In a survey of counselors who had experienced a client suicide, Wagner et al. (2020) found that a strong supervisory relationship at the time of a client suicide may help mitigate counselor survivors' negative symptoms. Survey participants were asked what type of agency responses were helpful to them after they had experienced a client suicide. A total of 55% indicated that receiving counseling was either extremely helpful or very helpful. Other helpful agency responses including allowing them to take time off from work after the suicide, debriefing discussions, and additional training.

For counselors who work with clients with substance abuse problems, the high relapse rate among these clients can lead to counselor discouragement and cynicism about the sincerity of clients' desires to change. Certain client characteristics such as lack of empathy for others, manipulativeness, and refusal to accept responsibility for one's behavior are often present in clients diagnosed with some personality disorders, sex offenders, and clients who abuse women or children. These characteristics also can exacerbate the stress felt by counselors. As has been noted, counselors who work with victims of trauma and abuse are at increased risk of developing secondary posttraumatic stress symptomology. Factors that can contribute to burnout and impairment among addictions counselors include heavy caseloads of mandated clients, staff turnover, funding cuts, low salaries, and limited career opportunities (Vilardaga et al., 2011).

The counseling profession has been slow to recognize the problem of impairment among its members; however, research and attention to the problem have increased in recent decades. In 2003, ACA created the Taskforce on Counselor Wellness and Impairment, which has organized and presented information on risk factors, assessment, wellness strategies, and resources, all of which can be accessed through the ACA's website (www.counseling.org). In a national survey of counselor wellness and impairment, Lawson (2007) found that counselors generally rated themselves high in wellness, but they rated the wellness of their colleagues significantly lower. A substantial number of counselors who participated in the study (5% to 10%) had scores on an instrument that indicated they were at risk for burnout and compassion fatigue or vicarious traumatization. More recently, Lawson and Myers (2011) surveyed more than 500 ACA members and found that counselors scored higher on the measure of wellness (Professional Quality of Life; ProQOL) than did populations previously studied, and they had lower scores on compassion fatigue and burnout.

The ACA *Code of Ethics* (2014) provides clear guidance regarding counselors who may be impaired. Standard C.2.g. states that counselors should monitor themselves for signs of impairment from their own physical, mental, or emotional problems. If they determine that they are impaired, counselors must limit, suspend, or terminate their professional responsibilities until they can safely resume their work and must seek assistance to address their problems (ACA, 2014).

RECOGNIZING IMPAIRMENT The ACA *Code of Ethics* (2014) also requires counselors to assist their colleagues or supervisors in recognizing their own impairment. If counselors believe that their colleagues or supervisors may be harming their clients, counselors must intervene as appropriate. It might be difficult for counselors to assist their colleagues and supervisors in recognizing their own impairment. At what point would you decide that a colleague or supervisor was impaired? For example, how much social drinking is too much? How many absences from work indicate impairment? Once you have formulated in your mind that a colleague or supervisor is impaired, how do you go about addressing the issue with them? Most individuals who are impaired go through an extended period of denial and probably would reject your input. In our opinion, this requirement in the *Code of Ethics* may be very difficult to comply with, especially if the counselor you believe is impaired is your supervisor, who has control and authority over you. Of course, you would need to intervene in some way to prevent harm to clients, but you should also be concerned that imminent harm will occur if you do not intervene and, if you do intervene, you must be prepared for a negative reaction. If you ever believe you need to confront a colleague or supervisor about your belief that they are impaired, this would be a good time to consult with other counselors or related professionals regarding your ethical obligations and to ensure that you have a personal support system as you move forward.

Ethical practice requires, first, that counselors be aware of warning signs of impairment. Some of the most common symptoms of impairment are presented in Figure 7-1.

FIGURE 7-1 Warning signs of professional impairment

1. Deterioration in personal relationships, including marital problems and family dysfunction
2. Becoming isolated and withdrawn from others
3. Feelings of disillusionment with the profession
4. Emotional distancing during counseling sessions
5. Poor health habits with respect to nutrition, exercise, and adequate sleep
6. Alcohol and drug abuse
7. Changes in work habits such as increased tardiness and absenteeism
8. Moodiness, depression, and symptoms of anxiety
9. Blurring of professional boundaries with clients
10. Procedural errors and sloppy recordkeeping

7-3 The Case of Suzanne

Suzanne is an LPC who went to work at a community agency when she earned her master's degree. After 5 years at the agency, Suzanne resigned and opened a private practice on her own. Now, 8 months into this venture, she isn't sleeping well at night. Suzanne tosses and turns, worrying about whether she will be able to pay the monthly bills and trying to think of new ways to market her practice. She misses the camaraderie she enjoyed with her colleagues at the agency and feels increasingly isolated. Suzanne has lost weight recently and is starting to have episodes of gastric distress. She realizes that she needs a break from the stresses of her work. However, because she feels trapped by her financial situation, she is taking on more clients during evening hours and on the weekends rather than taking time for self-care activities.

- What are some indicators that Suzanne is experiencing problems that might lead to impairment?
- If Suzanne does not change her behavior, what kinds of problems might you expect her to experience?
- If you were Suzanne's friend, what options might you suggest to her to relieve her distress?

Discussion: There are numerous indicators that Suzanne is experiencing problems that might lead to impairment: She isn't sleeping well at night, is worrying about whether she can pay her monthly bills and about finding ways to market her practice, misses her friends at her former worksite, feels isolated, has lost weight recently, is having gastric distress, and has taken on more clients than she really wants to serve.

If Suzanne does not change her behavior, she is at risk for impairment. The quality of her counseling services to clients may decline. Suzanne may develop additional psychological problems or physical health problems, and she may end up in a financial crisis.

If you were Suzanne's friend, you might suggest that she try to find another job and close her practice after finding a new job. Other suggestions might be that she could find one or more other counselors in private practice to share her expenses and provide Suzanne with professional stimulation, join a peer support or supervision group, or seek counseling for burnout management.

Perhaps the best way to prevent burnout and impairment is for individual counselors to monitor their own vulnerabilities and be aware of the ethical ramifications of practicing when they are unable to function effectively. Many resources and strategies are available to counselors when they recognize that they need to make changes in their lives if they are to remain competent practitioners (Kottler & Schofield, 2001). Some of these are (a) seeking counseling for themselves; (b) seeking supervision, especially of their work with clients who are difficult and tend to drain their personal resources; (c) taking a break or vacation from practice; (d) joining a peer support group; (e) getting regular exercise, meditating, and taking time to enjoy hobbies; and (f) seeking support from family and friends. Attending to self-care is an ethical obligation that must be taken seriously: Counselors cannot provide nourishment to clients if they do not nourish themselves (Corey et al., 2024; Culver, 2011; Patsiopoulos & Buchanan, 2011; Skovholt, 2012; Warren et al., 2010).

It is crucial that you develop self-monitoring skills so that you can stay psychologically and emotionally healthy as a counselor. Because graduate school is often stressful, now is a good time to learn and practice these skills (Roach & Young, 2007). Newsome et al. (2012) argued that counselor training programs should provide sufficient education about self-care and that counselor trainees should view self-care as an essential element of their professional development. Harrichand et al. (2021), noting the importance of self-care during stressful times like the COVID-19 pandemic, have provided specific recommendations for integrating self-care and wellness practices across the CACREP curriculum in counselor education training programs. If you are a student, we recommend that you learn the symptoms of burnout, familiarize yourself with various wellness models, and periodically take the time to assess how you are managing the stressors in your life. Corey et al. (2024) have suggested that you ask yourself some questions if you think you might be experiencing burnout. Your questions might include whether you ask supervisors or peers for help when you need it, whether you are willing to seek personal counseling if you think you need it (Cheng et al., 2018; Mullen & Crowe, 2017), whether you engage regularly in self-care practices, whether you are engaged in life outside of work, and whether you are staying connected to family members and friends.

DIVERSITY CONSIDERATIONS IN BURNOUT PREVENTION The results of a study conducted by Myers et al. (2003) on the wellness levels of counseling graduate students bode well for the future of the profession. These investigators found that counseling students scored higher than an adult norm group on the Wellness Evaluation of Lifestyle (WEL) and that doctoral students scored even higher than entry-level students.

Cause for concern in differences was found along cultural variables, however. Female students scored higher than male students on the measure of Gender Identity, and BIPOC students scored higher than White students on the measure of Cultural Identity. The researchers suggested that these findings may reflect greater awareness of gender and cultural issues on the part of persons who belong to historically oppressed groups in U.S. society. Still, the fact that doctoral students scored higher than entry-level students on gender and cultural identity subscales may indicate that counselor preparation programs are succeeding at developing awareness of diversity issues among their students.

Basma et al. (2021) found that wellness was negatively related to discrimination in counseling students who identified as Black, Indigenous, or people of color (BIPOC), suggesting that it may be more difficult for BIPOC students to maintain their personal wellness during their graduate studies. Challenges to maintaining wellness may continue after these students graduate. Shell et al. (2021) found that all three forms of racism (Individual, Cultural, and Institutional) that were measured on the Index of Race-Related Stress–Brief Version (IRRS-B) significantly predicted burnout for Black mental health therapists, including counselors.

ASSESSMENT

In Chapter 2, we noted that psychologists and other mental health professionals have challenged the boundaries of competence, or scope of practice, of counselors in two major areas: testing and diagnosis. Testing and diagnosis are both components of the more general professional activity of assessment, or evaluation. The ethical and legal pitfalls in the areas of evaluation, assessment, testing, and diagnosis are numerous (Welfel, 2016), and an entire section (Section E) of the ACA *Code of Ethics* (2014) is devoted to this topic. We begin this section with a discussion of ethical requirements and related legal issues in evaluation and assessment generally. Then we focus on testing specifically. Although diagnosis is a form of assessment, we discuss it separately because diagnosis involves a unique set of complex issues for counselors.

Evaluation and Assessment

COMPETENCE IN EVALUATION AND ASSESSMENT Despite arguments to the contrary made by psychologists and some other mental health providers, counselors have the education and skills necessary to conduct certain types of evaluations. Some counselors have jobs in which they conduct evaluations on a regular basis, and some have private practices in which they conduct evaluations occasionally or exclusively. To conduct an evaluation, a counselor should have expertise in the area that is being evaluated and must be prepared to serve as an expert witness in court. According to the ACA *Code of Ethics* (2014), counselors should utilize only those testing and assessment services for which they have been trained and are competent.

Counselors commonly conduct evaluations for a number of purposes. To give some examples, counselors evaluate:

- What type of custody arrangement would be in the best interests of a child (Patel & Choate, 2014)
- The extent of an individual's disability and the potential for future employment
- Whether an individual has a substance abuse problem and, if so, the type of treatment that is needed
- Whether a person is at risk for abusing children in the future
- Whether a person is at risk for harming others (Chambers et al., 2010; McCormac, 2014; Stutey & Clemens, 2014; Tillman et al., 2015; Winer & Halgin, 2016)
- Whether a person is at risk of serious self-harm or other self-injurious behavior (Buser et al., 2017; Donohue et al., 2015; Emelianchik-Key et al., 2016; Erickson & Abel, 2013; McGlothlin et al., 2016; Rutt et al., 2016; Stargell et al., 2018; Wester & McKibben, 2016; Wester et al., 2016, 2017; Whisenhunt et al., 2014)
- Whether a person has a diagnosable mental disorder
- Whether a person would perform well in a particular job

FORMAL EVALUATIONS Counseling most often involves face-to-face (or virtual) interviews between a counselor and client that are focused on the client's well-being and on assisting the client in personal decision making and problem solving. Evaluations completed by a counselor might include a personal interview, but interviews in which persons are being evaluated have a different purpose than counseling interviews. When a counselor who is also going to treat the client completes an evaluation, the sole purpose of the evaluation should be to gain information to allow the counselor and the client to understand the problems and establish appropriate counseling goals. Dual roles should be avoided (ACA, 2014, §E.13.c.).

Because counselors may sometimes be evaluators, and because evaluators usually conduct personal interviews as part of the evaluation process, it is understandable that individuals may become

confused about their interactions with evaluators. For example, a woman who goes to a counselor's office for an interview related to a child custody dispute may not understand that she is not entering into a counseling relationship, even though the counselor may be asking questions about life events that are very upsetting. The purpose of a child custody evaluation is to report findings to a judge or jury about which parent may be best to have custody of a child, not to assist the person being evaluated in dealing with the stressors of the situation. Child custody evaluations require that unbiased and objective recommendations be made to a judge (Ackerman et al., 2021). An evaluator must be objective and dispassionate. Counselors, on the other hand, always have the best interests of their clients in mind and, therefore, could not be objective evaluators if the findings might negatively affect their clients.

Before counselors begin an evaluation interview, it is essential that they fully describe the difference between being counseled and being evaluated. Counselors must ensure that the individual understands that this is not a counseling relationship and that the counselor may use any information gathered in the interview as the basis of a report and opinion that will be given to a third party. These obligations are described in the ACA *Code of Ethics* (2014). Standard E.13.a. clarifies that when counselors provide forensic evaluations, their primary obligation "is to produce objective findings" to be used in reports or testimony.

When an assessment or evaluation is completed, the assessment itself is the end product. A counselor or any number of other individuals might then use the assessment report for purposes other than for counseling. As noted, a personal interview of the person being evaluated is quite often a part of the process. Clients who are being assessed need to understand that the assessment process is different from counseling, what the assessment process entails, what the final product will be, and how and by whom the assessment is intended to be used. Counselors have a responsibility to secure their clients' informed consent to evaluation procedures. According to the ACA *Code of Ethics* (2014), the client's *written* informed consent should be secured before beginning a forensic evaluation (ACA, 2014). Before counselors conduct any type of assessment, they must explain its nature and purposes in language that the client can understand (ACA, 2014).

THE COUNSELOR AS EXPERT WITNESS Counselors should avoid conducting evaluations that cast them in the role of expert witness unless they have agreed to do so. If your job requires that you conduct evaluations or if you complete evaluations in your private practice, you must be prepared to testify in court as an expert witness. There is a good chance that eventually you will be subpoenaed and ordered to explain and defend your conclusions and recommendations.

Counselors also should be certain that they understand the circumstances when an attorney appears to be referring a client for counseling. Such referrals may be attempted for legal purposes as well as for clinical reasons, in that attorneys may be seeking a counselor who would later be subpoenaed to repeat in court what the attorney's client has said in counseling sessions, or the attorney may simply want to demonstrate that the client has gone through the motions of seeking counseling. Counselors should ascertain whether a referral from an attorney is related to an existing or a potential legal case and take this information into consideration when deciding whether to accept the client (Reid, 2010).

7-4 The Case of Alejandro

Alejandro has recently completed his master's degree in counseling and has taken a job in the probation and parole office at the local county courthouse. Alejandro's new supervisor has told him that his master's degree in counseling will be very helpful in his new position. Alejandro's job is to interview people after they have been convicted of a crime to determine whether they

are good candidates for probation or whether they should be put in jail or prison. Alejandro is told that good candidates for probation are those who have never committed crimes in the past or have had only minor violations, have a stable place to live, have jobs they have held for a while, and are truthful. Alejandro will look over the records of each convicted person, conduct an interview with that person, and then make a report to the judge, giving his opinion regarding whether the person is a good candidate for probation. In Alejandro's first interview with Tom, who has been convicted of a crime, Tom tells Alejandro that he is really glad to be able to talk to a counselor. Tom and Alejandro develop an easy rapport. In the course of the interview, Tom tells Alejandro about a number of crimes he has committed that did not show up on his criminal record and explains that he is about to become unemployed because of absenteeism, has recently been evicted from his apartment, and doesn't know where he is going to live. In court 2 weeks later, Alejandro reports to the judge that, based on information he has gathered from Tom, he does not believe that Tom is a good candidate for probation. Tom angrily tells the judge that Alejandro misled Tom into thinking he was his counselor and was trying to help him, and now Alejandro has betrayed Tom by telling the judge private information. Tom's lawyer makes a motion to have Alejandro's recommendation stricken from the record because Alejandro has no experience conducting probation evaluations and misled their client.

- When Alejandro began the meeting with Tom, what should Alejandro have said or done to prevent this unfortunate situation?
- How should counselors react if a person they are evaluating indicates during an interview that they believe this is a counseling session, even though the counselors have explained that it is an evaluation interview?

Discussion: Alejandro should have carefully explained to Tom the purpose of the interview and ensured that Tom understood the purpose before beginning. He also should have given Tom a written document to sign that reiterated these explanations. When Tom stated that he was glad to be able to talk to a counselor, Alejandro should have interrupted the interview and again explained his role as an evaluator. Alejandro should have called himself an *evaluator, probation consultant*, or some other title than *counselor*. He also should have explained to Tom that whatever Tom said could become a part of the report Alejandro is required to make and may be used in determining his probation decision.

An expert's testimony in court usually supports one side in a legal controversy and damages the other side (Patel & Choate, 2014). As a result, the attorney for the side that is damaged has a professional obligation to minimize the impact of the expert's testimony. The attorney will do this in one or more of the following ways: (a) object to the expert's credentials, arguing that the counselor is not expert enough to give an opinion in court; (b) attack the process the counselor used in completing the evaluation, claiming that it was not adequate; (c) attempt to get the counselor to make contradictory statements, hoping that the judge or jury will give less credibility to a witness who is not consistent; or (d) offer another expert witness's testimony that contradicts the counselor's conclusions and recommendations, hoping the judge or jury will give more credibility to the other expert's testimony. Aggressive cross-examinations can be challenging and uncomfortable for counselors who testify in court as expert witnesses (Brodsky & Terrell, 2011).

Counselors who agree to provide expert testimony in the belief that judges, attorneys, and juries will listen respectfully to the counselor's opinions and be appreciative of their input are in for

a big surprise. Some attorneys have argued that mental health, as a science, is too inexact to allow mental health expert testimony as evidence. Ziskin (1995) has produced a three-volume set of texts designed to assist lawyers in discrediting the testimony of mental health expert witnesses.

Counselors who plan to serve as expert witnesses should take courses to prepare themselves, observe other experts testifying over a period of time, and read written materials (e.g., Goldstein, 2016; Schouten, 2017; Stokes & Remley, 2001; Tedder-King & Czyz, 2016).

ASSESSMENT WITHIN THE COUNSELING RELATIONSHIP As described, counselor involvement in child custody determinations consists of a formal evaluation that is separate from the counseling process. Formal evaluations are different from the types of assessments that occur throughout a counseling relationship. In its broadest sense, assessment is the process of gathering information about a client (Leppma & Jones, 2013). Counselors use assessment to ascertain information about clients such as the nature of their problem; the severity and impact of the problem; the interplay among family, relationships, and past experiences with respect to the problem; the impact of environmental conditions on the problem; the client's challenges, strengths, and readiness for counseling; and whether counseling can be beneficial to the client (Drummond & Jones, 2010). Assessment is a collaborative process to which both the counselor and the client contribute, with the aim of gaining a better understanding of the client's problems. An accurate assessment of the problems, in turn, increases the likelihood that the problems will be resolved successfully in counseling. The most commonly used method for gathering this initial information is the unstructured clinical interview (Leppma & Jones, 2013). Other forms of assessment often are used within the counseling relationship to provide the counselor and client with more specific information than was gained during the initial interview. The most commonly used forms of assessment are tests of various types and diagnoses (Buser & Peterson, 2016; Buser et al., 2017; Carlisle et al., 2016; Jagger & Sterner, 2016). In the following sections, we first explore the ethical issues associated with testing.

Testing

Counselors use tests in various ways. Occasionally they may be involved in developing new tests, but more often they select existing tests, administer them, and interpret them for their clients. Selecting and administering a test may not seem like a complicated process that has ethical implications, but formal testing is vulnerable to abuse (Welfel, 2016).

7-5 The Case of Kamau

Kathleen has sought counseling from Kamau, LPC. Kathleen's job situation is tentative because the company where she has worked for the past 12 years is downsizing. She is experiencing a number of somatic complaints related to anxiety, along with a general feeling of apprehension. By the end of their second session together, Kamau believes that Kathleen may be experiencing

depression as well as anxiety. Because Kathleen's company's managed health care plan allows her only six sessions, Kamau doesn't want to use precious time for assessment activities. He thus gives Kathleen a standardized depression scale and tells her to complete it at home.

- What do you think of Kamau's decision to let Kathleen complete the test at home?
- Can you identify any problems with this procedure?

Discussion: Kamau should not have allowed Kathleen to take the test at home unless it was specifically designed to be self-administered (ACA, 2014, *Code of Ethics*, §E.7.d.). Counselors have a responsibility to appropriately supervise the test-taking process. Any number of things could happen that would produce invalid results if Kathleen takes the test at home. She might misread or misunderstand the directions, exceed the time limits for completing the test, try to complete the test while watching TV or being otherwise distracted, or become alarmed by feelings evoked by the wording of some of the test items.

COMPETENCE TO TEST Counselors generally do not use testing as much as psychologists do in their practices. The preparation that master's-level counselors receive in testing usually includes the following: (a) a three-credit assessment course, (b) some testing principles in a research class, (c) testing information in courses such as career counseling and diagnosis of mental disorders, and (d) supervised practice of the use of tests in practicum and internship. Because of the limited use that counselors generally make of tests and the educational level of counselors in the area of testing, it is important both ethically and legally for counselors to administer and interpret only the tests for which they have been trained and are competent (ACA, 2014, *Code of Ethics*, §E.2.a.).

At first glance, testing procedures might seem to be simple and straightforward. Test manuals give detailed, step-by-step instructions for administration, and many tests can be scored and interpreted by a testing service. In reality, however, considerable knowledge and professional judgment are needed to select the right test for the intended purpose and clientele, to administer the test properly, and to interpret the findings for the client. To select wisely, counselors must have a thorough knowledge of the various tests that are available, what they measure, and their limitations. Leppma and Jones (2013) have reminded counselors of their ethical responsibility to use *multiple* methods of assessment. This means that counselors should never use a test as the sole criterion for making clinical or educational decisions. To illustrate this point, even if Aaron (the counselor in the previous case example) had asked his client to complete the depression scale in his presence, Aaron should not use only the client's score on the scale to determine whether a diagnosis of depression is warranted. Aaron should also consider his clinical impressions and the client's behavioral indicators, consult the diagnostic criteria for depression in the *Diagnostic and Statistical Manual* (*DSM-5-TR*), and perhaps (with the client's permission) interview individuals who know the client well and have observed her general daily behaviors.

There is no absolute standard for adequate preparation to administer particular tests. Formal educational experiences such as for-credit courses and internships are not the only methods of training. Counselors could become proficient in testing through workshops, supervised practice, and independent study as well. The amount of training needed to develop proficiency varies greatly, depending on the test. Developing the necessary competence to administer complex personality tests such as the Minnesota Multiphasic Personality Inventory–2 requires extensive training. There are many tests, however, that counselors can learn to use through the instruction and supervised practice provided in a typical master's-degree program. Competence to use a test always depends on the individual counselor's training and experience with the particular test.

Although assessment, including the use of various tests, has always been an integral component of the counseling process, the question of who is qualified to use assessment instruments remains a contentious battleground (Watson & Sheperis, 2010). For this reason, an advocacy group, the national Fair Access Coalition on Testing (FACT), has been formed to protect counselors' rights to conduct assessment and testing (FACT, 2014).

As we noted in Chapter 2, counselor licensure laws now exist in all 50 states; however, some state laws either restrict counselors to certain testing activities or are ambiguous about whether counselors are qualified to make assessments. For example, some states prohibit counselors from using projective tests, and at least one state statute requires licensed counselors to be further *privileged* to test (Louisiana Licensed Mental Health Counselor Act, 1999). Counselors should review their state's licensing statutes and regulations to determine whether there are any tests they cannot legally administer and interpret. If licensed counselors exceed their statutory authority to test, they could be accused and found guilty of practicing psychology without a license. This happened to an LPC in Louisiana (*State of Louisiana v. Atterberry*, 1995).

Counselors should regulate themselves in the practice of testing. This is the position taken by the ACA, as explained in the *Standards for Qualifications of Test Users* (ACA, 2014). Although counselors typically do not have extensive training in testing, many resources are available to assist counselors in maintaining and increasing their competence to test. The website of the Association for Assessment in Counseling and Research in Counseling (AACRC), an ACA division, offers links to a variety of resources and testing documents. Two particularly useful documents are the *Code of Fair Testing Practices in Education* and the *Responsibilities of Users of Standardized Tests*. As the international counseling movement continues to grow, counselors who use tests in their work should familiarize themselves with the international guidelines for test use (International Test Commission, 2013).

DEVELOPING AND PUBLISHING TESTS Most counselors are not involved in developing testing instruments. Designing, developing, validating, and marketing a new test requires specific knowledge and skills and is a complex and lengthy process. These factors undoubtedly deter many counseling practitioners from undertaking the task.

Even if you are never involved in constructing a test, however, you should understand the ethical responsibilities of test producers. Test producers must produce and market tests that have demonstrated validity and reliability and appropriate norms, as well as provide a manual that clearly explains how to use the test, specifies the clientele for whom it is and is not suitable, and explains its strengths and limitations. They must also provide information that will help users interpret the results accurately.

Test publishers should make their tests available only to professionals who are qualified to use them. Typically, test publishers require those who wish to purchase their tests to state their degrees, licenses, graduate courses in testing, and training in the use of the specific test.

TEST SECURITY Counselors have both ethical and legal obligations to protect the security of tests from improper usage, copying, or dissemination (ACA, 2014, *Code of Ethics*, §E.10.). Almost all tests depend on the user being unfamiliar with the test items. Therefore, counselors must keep testing materials safely locked away when not in use to prevent unauthorized users from gaining access to the materials. Of course, test security measures must also be carefully maintained when the test materials are accessed and stored on a computer. A problem related to test security occurs when counselors and other professionals break the law by violating copyrights of tests held by test authors and publishers. The general principles of copyright law are explained in Chapter 16.

Test materials that are published by commercial testing companies must be purchased. Materials include answer sheets, question booklets, report forms, instruction manuals, and other items related

to the test. Computer software of all types related to testing is copyrighted and may not be duplicated or used without permission of the publisher. Some commercial testing companies are aggressive in protecting their copyrights because their business profits are negatively affected by violations.

Some tests have never been published by the author or may have been published in a journal. Such tests are not available from a commercial testing company. However, permission must be obtained from the copyright holder before counselors can use the tests either to administer to clients or for research. In most situations, authors transfer their rights to the owners of journals when materials are published.

RELEASE OF TESTING RECORDS Records of test results are the same as other counseling records. Counselors should thus not release test data to anyone who is not qualified to receive the information. The ACA *Code of Ethics* (2014) states that counselors should release test data only to persons recognized by counselors as competent to interpret the data. You may feel uncomfortable asking someone who requests test records to verify their qualifications, but releasing test data to unqualified individuals carries such a high risk to client welfare that counselors must stand firm on this point (Welfel, 2016). You also need to keep in mind that any time you share private client information with a third party, including a client's health insurance provider, that information is at risk for theft or exploitation. You will also need to maintain vigilance against threats to cybersecurity (Staton & Kielty, 2023).

Counselors also have an ethical obligation to ensure that testing records are transferred in a secure manner. Counselors who receive test results for clients they have been counseling should never transfer those records to another person. In the event that testing records received from another professional are requested or subpoenaed, counselors should refer the individual asking for the records to the professional who originally created them. Reviewing the section in Chapter 6 that deals with transfer of records in general will alert you to possible pitfalls and to recommended procedures for ensuring the safe transfer of test data.

PROVIDING EXPLANATIONS TO CLIENTS Clients have the right to make an informed choice about whether they want to participate or refuse to engage in testing procedures. They can also expect to receive an explanation of the results of any test they take.

Standard E.3.a. of the ACA *Code of Ethics* (2014) clearly indicates that informed consent applies to testing. Before counselors test, they must explain the nature and purposes of assessment and the specific use of the results in language that the client can understand. It is particularly important that clients understand what uses will be made of their test results. They have a right to know in advance if tests will be used as a factor in employment decisions, as a criterion for placement in a special education program, as a device for screening potential members of a psychotherapy group, or as a means of making other decisions that will affect their lives (ACA, 2014, §E.3.a.).

After clients have participated in testing procedures, they have a right to receive feedback about the results (ACA, 2014, §E.1.b.). Many standardized inventories are now computer scored, and counselors can receive results along with printouts that present an extensive explanation of those results.

7-6 The Case of Louis

Louis is a counselor in a university career counseling and placement center. Last week he began counseling Alicia, a freshman who sought his assistance in deciding which major to pursue. In their first session, Louis explored with Alicia her sense of being overwhelmed with so many interesting options. After the session ended, Alicia sat at a computer in the next room and

completed a career interest inventory. Before his second session with Alicia, Louis obtains her computer-generated score report and interpretation. Louis reads it over, is impressed with its thoroughness, and decides to give it to Alicia to read at home. Louis decides in favor of using their time together to explore some other issues Alicia has mentioned, which seems to Louis to be related to Alicia's career indecisiveness.

- What are the problems with giving a client computer-generated test results without also providing an interpretation of the results?
- In what types of situations might you ethically use computer-based testing with clients?

Discussion: Although it is easy to understand Louis's reasoning, it would not be wise for him to follow his plan. Alicia might think *the computer has given her the answer* and may attribute too much power to the test results. Using computer-based testing with clients is not problematic as long as the counselor interprets the results for the client in person and puts the results in the context of other information about the client.

It is essential that counselors communicate test results and interpretations directly to their clients. Counselors appreciate commercial services that score and interpret psychological tests because the services save time and relieve them of the task of scoring the tests, but they should avoid overreliance on them. Interpretations generated by commercial services cannot, by themselves, provide sufficient feedback to clients. They do not take into account the individual characteristics of the test taker and cannot answer any questions the test taker may have. Counselors must therefore exercise their professional judgment and personal knowledge of the client in using reports provided by test interpretation services.

A face-to-face conversation with the client regarding test results can serve additional purposes. Clients sometimes view test results—particularly elaborate, computer-generated score reports—as being infallible. Counselors need to educate clients about the limitations of tests and to keep these limitations prominent in their own thinking. This will help them avoid any tendency to present results in absolute language. In addition, a dialogue with the client can shed light on any unclear or unexpected test results.

For some clients, taking a test is an anxiety-producing event, and they may worry until they see their results. Counselors should keep in mind that some clients have had negative experiences with test taking. On the other hand, feedback can have therapeutic value. Finn and Tonsager (1992) found that clients who received feedback on their Minnesota Multiphasic Personality Inventories (MMPI) found the feedback to be a positive experience and actually showed a reduction in symptoms.

In practice, counselors often struggle with the question of how much feedback or explanation of test results they should give their clients. They want to be thorough, without overwhelming the client with detail. For many counselors, the pressures of time-limited counseling relationships imposed by managed care are also a factor. Welfel (2016) has suggested that the criteria for deciding how much time to devote to test interpretation should include the following: (a) the clients' satisfaction that they understand the meaning and implications of the results, (b) the counselor's assessment that feedback has clarified any confusion or unclear results, (c) a mutual agreement about ways the test results should influence the counseling process, and (d) the implications of releasing the findings to others if the client has agreed to such a release.

Diversity Issues in Testing

Counselors must be cautious in using tests with culturally diverse clients. They must select tests carefully to prevent inappropriate testing that may lack appropriate psychometric properties for a particular client population (ACA, 2014, §E.8.). It is important to be aware that many traditional assessment instruments have been developed by theorists and practitioners who are themselves lacking in diversity and multicultural training (Helms & Cook, 1999). Because many tests have been validated on mostly middle-class people of European extraction, special populations may not be represented in the norm group on which a test was standardized. Therefore, the test results may be less valid for them. When you use tests, you need to be knowledgeable about the characteristics of the norm group and recognize the limitations of the test with other populations.

The ACA *Code of Ethics* (2014) requires counselors to recognize the effects of age, color, culture, disability, ethnic group, gender, race, language preference, religion, spirituality, sexual orientation, and socioeconomic status on test administration and interpretation. Especially when working with culturally diverse clients, test results must be placed in perspective with other relevant factors (ACA, 2014).

No test is completely culture free. For decades, a considerable body of literature was produced that provided examples of how assessment instruments have been misused with ethnic and racial minority clients, women, clients with limited English proficiency, the physically challenged, and other special populations (Herlihy & Watson, 2003). The use of educational and psychological tests, in the school setting in particular, has been challenged. Today, there is some evidence that progress has been made in addressing this problem. Reynolds et al. (2021) traced the history of Cultural Test Bias Hypothesis (CTBH) and found that, while more research is necessary, the current evidence tends to support the proposition that most commercially developed, widely used tests of achievement and aptitude are not culturally biased. Nonetheless, academic placement or advancement decisions should never be made on the basis of test data alone. Counselors need to know about the norm groups on which tests are standardized and examine tests for content bias. When some items in a test refer to experiences that are not familiar to certain populations, the test is content biased. For example, an item on a test that refers to milking cows may make no sense to an urban child who lives in an impoverished socioeconomic environment. Counselors should thus always review any tests they plan to use to ensure that the tests are sensitive to culture-specific variables associated with racial and ethnic minority group members' experiences.

Counselors must be alert to the passage of laws that affect testing practices. Two examples are the Education for All Handicapped Children Act (1975) and the Americans with Disabilities Act (1990). The Education for All Handicapped Children Act requires that tests be in a child's language and be appropriate for the intended use. It also requires that parents give their consent to testing and that they be given access to test protocols after testing has been completed. The Americans with Disabilities Act mandates that testing be appropriate for the individual being tested, that reasonable accommodations be made in testing, and that results not be used to discriminate in employment choices for people with disabilities.

In summary, counselors must be alert to cultural bias in tests and in test interpretation and must use caution in interpreting results with clients who are not members of the dominant culture. Ethical practice requires counselors both to understand how clients' cultures and worldviews can influence their performance on standardized tests and to place test results in perspective with other factors in evaluating these clients.

DIAGNOSIS

Counselors are sometimes uncomfortable with participating in the diagnosis of mental disorders. Diagnoses usually are made using the *DSM-5-TR* (APA, 2022), which is the world's most widely used system for identifying and describing mental disorders (Gray, 2011). This system of diagnosis is based on the medical model of mental illness. In contrast to the medical model, counselors espouse a wellness model that focuses on client strengths (Slaiba & Barden, 2017) rather than on simply ameliorating symptoms (see Chapter 2 to review the wellness model). Thus, there is an inherent philosophical conflict between the *DSM* system and the wellness orientation of counselors (DeKruyf et al., 2013; Frances, 2014; Kress et al., 2010).

During the 1990s, a lively conversation took place in the professional literature regarding whether counselors, with their wellness perspective, should diagnose clients with clinical disorders using the *DSM* system. Today, it is generally understood that counselors, no matter where they work, must be knowledgeable of the *DSM-5-TR* so that they can participate in the diagnostic process and be able to converse with other mental health professionals regarding its contents (Dailey et al., 2014a; Gutierrez et al., 2018). The CACREP standards (2024) specify that counseling students must acquire the competencies needed to effectively use the *DSM-5* in their work, and many state counselor licensure boards require applicants to demonstrate formal training in diagnosis.

When clients want to use their health insurance to help pay for mental health care, counselors usually must assign a diagnosis in order for their services to be reimbursable. As we discuss more fully in Chapter 13, health insurance companies generally will not reimburse for counseling services unless a *DSM* diagnosis accompanies the insurance claim (Comer & Comer 2021). Counselors want to serve their clients by helping them receive reimbursement for counseling services, yet their philosophical objections make them reluctant participants in diagnosis. Braun and Cox (2005) have warned that discomfort with diagnostic labeling can lead to problems: Some counselors may try to diagnose most or even all of their clients as having an adjustment disorder, which is the mildest and least stigmatizing type of disorder. Some might assign a more serious diagnosis than is clinically warranted so that a client can qualify for health insurance reimbursement. These practices of under-diagnosing, or *downcoding*, and over-diagnosing, or *upcoding*, are dishonest, inaccurate, and unethical (Herlihy et al., 2008). Even more serious is the practice of deliberately changing a diagnosis for insurance reimbursement purposes. Over two decades ago, some researchers found that approximately 35% to 44% of mental health practitioners may have been engaging in this behavior (Danzinger & Welfel, 2001; Tubbs & Pomerantz, 2001). Today, this practice is likely less widespread, as multiple computerized systems for detecting health insurance fraud are being utilized and as counselor training emphasizes the importance of honest and accurate diagnosis. Clients can be harmed by inappropriate diagnostic procedures, and counselors are on shaky ground both ethically and legally by engaging in such procedures. Standard E.5.a. of the ACA *Code of Ethics* (2014) states that "counselors [should] take special care to provide proper diagnosis of mental disorders."

Functions of Diagnosis

It is vital that you use the *DSM-5-TR* appropriately in your work as a counselor. A first step is to clarify your own stance toward diagnosis. Diagnosis involves much more than assigning labels to clients. It is best conceptualized in context, as a part of the overall process of assessment that helps counselors understand their clients. Sleek (1996) has likened the diagnostic process to working a jigsaw puzzle in that it involves putting together pieces of information about the client to build an overall picture of the person. The primary purpose of diagnosis is to facilitate effective treatment. The relationship between diagnosis and treatment is like a driving trip and an app that shows the route and provides directions

for arriving at the destination. Diagnosis should point the way to treatment planning and to the selection of effective treatment strategies for identified disorders (Gutierrez et al., 2018).

Diagnosis serves other important functions. First, an accurate diagnosis can help counselors make a prognosis or predict the course of a disorder. Second, the diagnostic system in mental health care was developed to facilitate communication among professionals and provides a common language and shared frame of reference for mental health professionals. Third, it helps counselors identify conditions that may require the attention of a physician, such as organic disorders or medical conditions that are contributing to psychological problems, so that a referral can be made. Finally, the diagnostic system provides a framework for research. Clinical outcome studies build our knowledge base regarding the effectiveness of various treatment approaches.

Risks of Diagnosis

While keeping in mind all the useful functions of diagnosis, you should be aware of the risks involved, even when diagnosis is done honestly and accurately. For clients, learning their diagnosis can be a double-edged sword: It can be comforting for clients to learn that their condition has a name and that they are not alone in what they are experiencing. On the other hand, clients may feel embarrassed or ashamed of their diagnosis or feel stigmatized by a diagnostic label and refuse treatment rather than have the diagnosis become part of their health record. It is also possible that a diagnosis can lead to a self-fulfilling prophecy; for instance, a client who is diagnosed with depression may become even more deeply depressed. Counselors must therefore be sensitive to the powerful effect that diagnosis can have on clients. The ACA *Code of Ethics* (2014) allows counselors to "refrain from making and/or reporting a diagnosis if they believe it would cause harm to the client or others." In most circumstances, clients have access to their counseling records, including any diagnoses that counselors have rendered.

Diagnoses can significantly affect not only clients' feelings and self-concepts but also their lives. Severe diagnoses can have harmful effects far into a client's future, including difficulty obtaining certain kinds of employment or having their diagnosis used negatively in child custody proceedings (Overton & Medina, 2008). Wylie (1995), in a widely quoted statement, noted that some diagnoses can "shadow a client's life like hounds from hell" (p. 32).

The process of diagnosing a client's condition can have effects on the counselor as well as on the client. It is possible that the very availability of diagnostic labels and agency or organizational pressure to use them can bias counselors in favor of using them, even when a client's condition does not warrant a *DSM* diagnosis. Particularly when a client's eligibility for health insurance coverage depends on a diagnosis, counselors may be tempted to pathologize behaviors that are developmentally normal. A rebellious adolescent might be diagnosed as having *oppositional defiant disorder* or *conduct disorder*. A very shy young adult might be diagnosed as having an *avoidant personality disorder*. Welfel (2016) has cautioned counselors against jumping ahead to naming a problem before they have verified that one exists. Carrola and Corbin-Burdick (2015) expressed the concern that a myopic focus on symptoms and diagnoses can further stigmatize certain clients (for example, assessing for PTSD when counseling military veterans and thus failing to consider their unique experiences and needs). We agree that counselors who use diagnosis must take care to avoid losing sight of the uniqueness of each client. Even while using a system that is focused on mental disorders and pathology, counselors must maintain the wellness perspective that is unique to their profession. Doing so can be a challenge, however.

Ethical Challenges in Diagnosis

The *DSM-5* was published in 2013 after a lengthy revision process. Considerable controversy emerged when it was discovered that 69% of the task force members who oversaw the development of the manual had ties to pharmaceutical companies (Cosgrove & Wheeler, 2013). That situation raised

concerns that client behaviors and cognitions that were formerly considered unremarkable would be used to justify diagnoses of mental disorders that could be treated pharmacologically (Blumenthal-Barby, 2014). On the positive side, the ACA offered input regarding proposed changes throughout the revision process, and the leadership applauded the changes, such as reversing the pathologizing of normal bereavement, that have reduced the dissonance between the *DSM* and the wellness orientation of counselors. We believe it was important for ACA to have offered input into the revisions as a means to establish the parity of professional counselors with other mental health professionals in the realm of diagnosis. Nonetheless, many of the changes made in 2013 were controversial, both among counselors and within the psychiatric profession, and the *DSM* system remains firmly grounded in the medical model. With the more recent publication of the *DSM-5-TR*, 2022 there are no DSM codes; only ICD-10-CM (International Classification of Diseases, 10th Edition, Clinical Modification) are used. This change more firmly cements the *DSM* system in the medical model. Some ethical issues that arise for counselors when they use diagnosis are securing the client's fully informed consent, working with physicians, and having the qualifications to diagnose. Multicultural considerations are also important in diagnosis. Following is a discussion of these issues as well as legal issues related to accurate diagnosis.

INFORMED CONSENT Some clients do not understand the stigma that might be associated with being diagnosed with an emotional or mental disorder, whereas others may be very concerned about the implications of diagnosis for their future employment. Although it would be inappropriate to overemphasize the possibility that a client may be discriminated against in the future as a result of receiving a diagnosis, counselors do have a responsibility to raise the issue with clients. The challenge is to present a fully adequate explanation of the diagnostic process without deterring the client from seeking counseling services (Kress et al., 2010, 2013). See Chapter 4 for a fuller discussion of informed consent and diagnosis.

When clients are participants in managed care plans, considerable information must be discussed. Clients must understand the impact that their insurance policy may have on the types and length of treatment that will be reimbursed. They must be aware that third-party payment might end before they believe they have met their counseling goals. They must also know what and how much information will be released to managed care companies, and they must give the counselor permission to release the information. The ACA *Code of Ethics* (2014) specifies that "counselors [should] disclose information to third-party payers only when clients have authorized such disclosure."

So that clients cannot later claim that they did not realize they might receive a diagnosis of an emotional or mental disorder, counselors' disclosure statements should include a phrase regarding diagnosis. We also recommend that, whenever possible, counselors should disclose to clients (or their parents or guardians) their diagnosis when the diagnosis is recorded. If an initial diagnosis is changed after the counseling process has begun as a result of a counselor's reassessment of the client's condition, of course this change should also be discussed with the client. As we note in Chapter 4, informed consent should be viewed as an ongoing process, not a one-time event. This may be particularly important to remember with respect to diagnosis. Grover (2005) has cautioned that some clients may not adequately understand at the time of their initial consent the full implications of being assigned a diagnosis and having it communicated to others.

CONSULTING WITH PHYSICIANS Counselors have a responsibility to understand that some client behaviors or reported thoughts and feelings could have a physiological basis. Counselors are wise to determine the physical condition of their clients when they begin the counseling process. Intake forms or interview protocols must ask clients whether they have any negative health conditions and, if so, the details regarding their treatment of those conditions. Counselors must also know whether their clients are taking any medications and, if so, the purposes of the medications and the benefits and side effects that the medications produce (King & Anderson, 2004). When clients have not had a physical exam in a long time, or if a possibility exists that a problem may be physical in nature, counselors should recommend that clients have a physical exam. They should also ask the clients to report the results to them or to sign a release of information so that the counselor and the physician can communicate with each other.

When clients have physical complaints that obviously might be related to medical problems (such as headaches, dizziness, or chest pains), counselors must insist that these clients have physical examinations as a condition to their continued mental health treatment. Mental health professionals who overlook or ignore the obvious need for a referral to a physician could be held negligent if a client's supposed mental condition later turned out to be caused by a physical problem.

The symptoms of some mental disorders can be alleviated by psychotropic medications. For example, several medications have been demonstrated to be effective in reducing symptoms of bipolar disorder (formerly called *manic depression*) in many sufferers. When counselors diagnose conditions that can be helped by taking medication, they must make clients aware of this option. Counselors can then refer to a psychiatrist those clients who are willing to consider taking medication. With the client's permission, the counselor and the psychiatrist can work cooperatively in assisting the client.

Qualifications to Diagnose

An issue that has been controversial is whether counselors are qualified to diagnose mental disorders. Of course, counselors should not assign a diagnosis of an emotional or mental disorder unless they have had adequate training to do so (ACA, 2014, *Code of Ethics*, §C.2.a.), but there is no absolute standard for determining adequate training. In terms of academic preparation, most contemporary programs that prepare counselors for clinical mental health or community agency practices require counselors to complete a course in the diagnosis of emotional and mental disorders. Counselors should also have had supervised experiences in diagnosing as part of their practicum or internship in their master's-degree programs, should continue to gain supervised experience in diagnosis while they are working toward their licenses, and should seek ongoing consultation and supervision of their diagnostic practices (Kress et al., 2010). New research about effective treatments is appearing constantly, so counselors must continue to learn in order to stay current (ACA, 2014, *Code of Ethics*, §C.2.f.).

In some states, licensed counselors have been challenged and accused of practicing outside their scope of authority when they have diagnosed emotional and mental disorders. The *DSM-5-TR* (2022) acknowledges that the diagnosing of mental disorders is applicable in a wide diversity of contexts and is of value to all professionals associated with various aspects of mental health care. These professionals include psychiatrists, other physicians and health professionals (including psychologists, counselors, nurses, and occupational and rehabilitation therapists), social workers, forensic and legal specialists, and researchers (APA, 2022, *DSM-5-TR*).

Accusing counselors of practicing outside their scope of authority when diagnosing emotional and mental disorders is an example of a *turf issue* that can pit counselors against other mental health professionals such as psychiatrists, psychologists, and social workers, who have traditionally endorsed the medical model. This issue is very important because if mental health professionals are unable to render diagnoses of emotional and mental disorders, some agencies may refuse to employ them, and health insurance companies may not reimburse their clients for mental health services that counselors provide.

Counselors have responded to this problem by introducing legislation that amends the scope of practice in their state licensure statutes (Kassirer et al., 2013). The amendments specifically indicate that licensed counselors may diagnose and treat emotional and mental disorders. In states that do not have such specific language in their counselor licensure statutes, counselors still diagnose emotional and mental disorders. However, they must argue that doing so is part of the counseling process in general that is covered in other language in their counselor licensure statutes.

Diversity Considerations in Diagnosis

It is important to keep in mind when dealing with diagnosis that *mental disorders* are defined in a cultural context. Standard E.5.b. of the ACA *Code of Ethics* (2014) states, "Counselors [should] recognize that culture affects the manner in which clients' problems are defined and experienced. Clients' socio-economic and cultural experiences [should be] considered when diagnosing mental disorders." Behaviors that may seem bizarre in one culture may be considered perfectly normal in another.

The *DSM* system has long been criticized for pathologizing normal behaviors and for bias against women, members of ethnic and racial minority groups, geriatric populations, children, people living in poverty, and LGBTGEQIAP+ individuals (Rapp & Goscha, 2012; Zur & Nordmarken, 2010). In the *DSM-5* (American Psychiatric Association, 2013), an attempt was made to give more attention to the diverse ways in which people from different cultural groups can experience and describe their distress (Dailey et al., 2014b). A section on *cultural formation* was expanded to include cultural syndromes, idioms, and explanations. Nonetheless, this cultural formulation fails to fully address the concerns of wellness-oriented counselors because it focuses on pathology rather than on client strengths (Comaz-Dias & Brown, 2016).

Concern has also been expressed that the *DSM-5* may not be appropriate when working with traumatized populations such as refugees (Goodman et al., 2018). Assessment and treatment based on a Eurocentric diagnostic system may pathologize cultural expressions of distress. When refugees have been exposed to historical or intergenerational trauma that has been passed from generation to generation, it is even more imperative that comprehensive and ongoing trauma assessments be made, rather than relying solely on the *DSM-5* (Goodman et al., 2017).

In an attempt to address these concerns, the task force that conducted the revisions that appear in the latest edition, the *DSM-5-TR* (2022), considered that "sociocultural diagnostic contexts have been historically under-emphasized in assessment, diagnosis, and treatment" (Wilson, 2022, p. 11480). Four cross-cutting review groups (of culture, sex and gender, suicide, forensics) reviewed all chapters. The text was also reviewed by a work group on ethnoracial equity and inclusion to ensure appropriate attention was given to risk factors such as racism and discrimination and the use of non-stigmatizing language (APA, 2022, p. 1). The *Cultural and Social Structural Issues* section includes subsections on the impact of cultural norms and practices, cultural concepts of distress, the impact of racism and discrimination on psychiatric diagnosis, and attention to culture, racism, and discrimination (APA, 2022). The *DSM-5-TR* also provides clinicians with updated and expanded information on how sociocultural issues may impact how individuals present with specific disorders. Nonetheless, discussion about the impact of other aspects of identity (e.g., gender identity, religion, sexual orientation, class, ability) and the intersectionality of identities is lacking. Overall, work remains to be done to make the *DSM* inclusive and affirming (Wilson, 2022).

Despite attempts to increase cultural sensitivity, it is important to remember that the *DSM* only *describes* mental disorders. It does not consider the etiology, or possible origin, of mental disorders. The manual does not ask clinicians to consider, for instance, why twice as many women as men are diagnosed with major depressive disorder. Is it because women are more prone to depression than are men? Is it because it is generally more acceptable in our society for women to express sadness and to

ask for help for symptoms of depression? Is it because women are more likely as an oppressed group to experience feelings of hopelessness and depression? Could it be due to other reasons? Several studies have demonstrated gender bias against women among mental health professionals, including mental health counselors (Schwartz et al., 2011).

Herlihy and Watson (2003) identified three ethical problems in the use of the *DSM* system with diverse clients. First, the *DSM* is based on the medical model of mental illness, which defines problems as residing within the individual. Failure to adequately consider social, political, economic, and cultural factors in clients' lives does a disservice to clients whose problems originate in or are exacerbated by oppression, marginalization, and discrimination. This issue has been addressed to some extent in the most recent revisions made in 2022.

Second, the *DSM* system tends to pathologize the problems of racial and ethnic minority clients, as well as women, and adds to the stigma and oppression of individuals from nondominant groups (Banks-Van Allen, 2023). A number of studies have demonstrated that members of minority groups tend to receive more severe diagnoses than members of the majority culture for the same symptoms. Studies have shown that African Americans are more likely to be diagnosed as suffering from schizophrenia (Neighbors et al., 2002, 2003), childhood disorders (Feisthamel & Schwartz, 2009), and major depression (Gara et al., 2019) than are Americans of European descent. When compared to non-Hispanic White youth, some ethnic and racial minority youth are more likely to receive a diagnosis of a disruptive behavior disorder and are less likely to receive a diagnosis of ADHD (Fadus et al., 2020); sexual minority individuals have been found to be more likely to be diagnosed with borderline personality disorders than are heterosexual individuals (Rodriguez-Seijas et al., 2021). Hispanics have been diagnosed with schizophrenia at a rate 1.5 times higher than Whites; LGBTGEQIAP+ individuals are diagnosed at a 3-to-4.7-times higher rate for major depression and panic disorder; women are diagnosed disproportionately with personality disorders; and clients with lower educational levels are more often diagnosed with schizophrenia than with mood disorders (Levers & Hyatt-Burkhart, 2020). These practices continue; Garb (2021) conducted a systematic review of existing literature and concluded that race and gender bias are present across numerous mental health diagnoses.

Third, the *DSM* system tends to perpetuate a paternalistic approach to mental health care and thus to reinforce the societal oppression of women and minority clients. The medical, pharmaceutical, and psychotherapy industries profit tremendously from the treatment of mental disorders (Herlihy & Watson, 2003; Rave & Larsen, 1995). Women receive more prescriptions than do men in all classes of drugs, particularly psychotropic medications. The medical model also mirrors the dominant/subordinate relationships between the authority who diagnoses and prescribes and the women and minority culture clients who are the recipients of these services (Feisthamel & Schwartz, 2009; Rapp & Goscha, 2012). Counselors who strive to avoid replicating in the counseling relationship the inequities experienced by women and minority clients will reframe many symptoms as coping mechanisms and evidence of survival skills in an oppressive culture rather than as evidence of pathology.

The ACA *Code of Ethics* (2014) reflects the concern of counselors regarding historical and social prejudices in the diagnosis of pathology. Standard E.5.c. states that counselors should recognize these prejudices that have led to the misdiagnosing and pathologizing of certain individuals and groups and should "strive to become aware of and address such biases in themselves or others" (ACA, 2014).

Despite the problems inherent in the *DSM* system and despite the discomfort of many counselors in using this system, it would be impractical for counselors to refuse to participate in the *DSM* diagnostic process. Rather, counselors have an ethical responsibility to reframe their thinking about the *DSM* system in more culturally competent ways (Herlihy et al., 2008; Zalaquett et al., 2008). Counselors must be careful to maintain their wellness and holistic orientations and to avoid

equating clients with their disorders via applying diagnostic labels. For example, a client should never be referred to as *a schizophrenic* but, rather, described as *an individual who suffers from schizophrenia* (Dorre & Kinnier, 2006).

Legal Issues in Diagnosis

Earlier in this section, we made the point that diagnoses rendered by counselors must be accurate and honest despite any philosophical uneasiness or cognitive dissonance they may be experiencing. We noted that some mental health professionals record what they consider to be less serious disorders than what they believe the client has in an effort to prevent stigma toward the client. At the other end of the spectrum, counselors sometimes assign diagnoses that are more serious than warranted so that their clients will receive third-party reimbursement for counseling services. These practices are not uncommon. However, inappropriate use of a diagnostic category to obtain insurance reimbursement is the type of financial misconduct that is frequently brought before ethics committees, licensing boards, and courts.

7-7 The Case of Charlotte

Charlotte has been a counselor in a group private practice for 6 years. Charlotte shares office space with other counselors but maintains her own separate private practice business. Most of the managed health care companies for which Charlotte is an approved provider require the identification of an emotional or mental disorder before they will agree to pay for Charlotte's counseling services. Most companies require that Charlotte diagnose each client using the *DSM–5-TR*. *Other Conditions That May Be a Focus of Clinical Attention*, sometimes called *V-Codes*, are not acceptable. Charlotte, having been educated to counsel clients based on a wellness perspective of mental health, worries about labeling clients negatively by diagnosing them with mental disorders. In reviewing the *DSM–5-TR*, Charlotte has decided that "adjustment disorder" is the least objectionable diagnosis and is the least likely to distress clients if she diagnoses clients with it. Even though many of her clients do not meet the technical criteria for receiving a diagnosis of adjustment disorder, Charlotte uses that diagnosis anyway. About 80% of Charlotte's clients are diagnosed with adjustment disorder.

- Do you agree with Charlotte's diagnostic practices? Why or why not?
- How can a counselor with a wellness perspective justify diagnosing clients with *DSM–5-TR* mental disorders?

Discussion: Charlotte's motives in diagnosing most of her clients with adjustment disorder are understandable, but her actions are not ethical and also constitute insurance fraud because she is lying to managed care companies to collect payments from them. Charlotte can thus be sued by the companies and charged with a crime by a prosecutor. Charlotte must therefore make the effort to reconcile her wellness perspective with using the *DSM* system so that she diagnoses honestly and accurately.

Practices such as those Charlotte is using are identified in Chapter 13 as insurance fraud, which can expose counselors to both civil and criminal liability. A client could bring a civil malpractice suit against the counselor, and a prosecutor could bring criminal charges against the counselor based on the fraud. A client who was assigned a false diagnosis would have an excellent foundation for a successful malpractice suit against the counselor if the client later lost a job, was denied a license, or was denied a security clearance because of the false diagnosis. If it could be shown that a mental health professional knew or should have known that a client had a serious disorder but recorded a less serious disorder and if that client then received inadequate treatment that led to harm or was denied health insurance reimbursement because of the false diagnosis, the mental health professional could be found to have committed malpractice. Obviously, you want to prevent your ever having to face such a situation. It is thus crucial that you do not engage in the practice of assigning a diagnosis to fit insurance companies' criteria for reimbursement, even if you observe others doing so or hear justifications for such fraudulent practices.

Summary and Key Points

This chapter discussed three main issues: counselor competence, conducting assessment and testing, and assigning diagnoses.

Counselor competence is an important concept, even though it is difficult to define from an ethical viewpoint and difficult to demonstrate in a court of law. It is best viewed as being based on performance and as existing along a continuum from gross negligence to maximum effectiveness.

- Counselors must practice within their boundaries of competence. It can be difficult to determine just where these boundaries lie.
- The development of competence begins with preparation and education, and the initial responsibility for producing competent practitioners rests with counselor educators and supervisors.
- Preparation standards established by accrediting bodies help to ensure that graduates of accredited programs possess certain competencies. Nonetheless, graduation from an accredited counselor education program does not guarantee competence.
- Licensure is a legal process that establishes minimum standards for a counselor to be able to practice in a given state. Because licensure is a political process, counselor licensure requirements are not uniform across the states. Certification is another approach to attempting to ensure competence.
- Once counselors begin to practice, they are responsible for determining their own competence.
- Counselors must know when and how to refer clients when they are not able to provide competent services to these clients. Referrals must not be made on the basis of value conflicts; rather, referrals should be based on a lack of competence to work with a client.
- Counselors must exercise care while stretching their boundaries of competence to include client populations and concerns with which they have little or no experience.
- It is essential in today's society that counselors possess intercultural counseling competence.
- When counselors are experiencing distress, burnout, or impairment, they must take steps to protect their clients from harm and to restore themselves to their full level of functioning.

Assessment and evaluation are collaborative processes between counselor and client during which they work together to gain a better understanding of the client's problems. An accurate assessment then guides the treatment planning. Testing is a specific type of assessment.

- Counselors are competent to conduct many types of evaluation.
- Evaluations must be objective, and for this reason, counselors should avoid taking on dual roles, such as counselor and child custody evaluator.
- Clients have a right to understand what an assessment process will involve, what its purposes are, and the uses to which it will be put.
- When counselors conduct evaluations, they must be prepared to serve as expert witnesses in court.
- Counselors are responsible for maintaining the security of tests that they use and for appropriately supervising the test-taking process.
- Counselors are competent to administer and interpret many tests, but they should keep in mind that some tests require specific training in their use.
- Mental health professionals in some states have attempted to prohibit counselors from using certain kinds of tests.
- Counselors must use caution in releasing test records to third parties and should release the records only to those individuals who are qualified to receive them.
- Clients have the right to receive a full explanation of their test results.
- Counselors must be cautious in using tests with culturally diverse clients. They should check to determine whether individuals similar to their clients are represented in the norm groups on which the tests were standardized.
- Counselors must be knowledgeable about laws that affect testing practices.
- Diagnosis is a form of assessment that raises difficult questions for counselors, primarily because counselors espouse the wellness model. Diagnosis has caused considerable controversy historically, and concerns about bias in diagnosis continue to exist.
- Counselors are often uncomfortable with diagnosing mental health disorders because the prevalent diagnostic system is based on the medical model.
- Clients must understand the implications of any diagnosis that might be assigned to them.
- Counselors must be prepared to work cooperatively with physicians when clients may have a physical condition that is contributing to their mental or emotional problems or when they could benefit from taking medications for their condition.
- A controversial issue has been whether counselors are qualified to diagnose.
- Multicultural considerations are of paramount importance in diagnosis because all mental disorders occur in and are defined by a cultural context.
- Counselors must be alert to sources of bias against non-dominant members of society in the diagnostic process and must take care not to perpetuate historical and social prejudices in diagnosing pathology.
- Counselors are guilty of insurance fraud when they do not diagnose honestly and accurately, and they can be subject to both civil and criminal liability.

Malpractice and Resolving Legal and Ethical Challenges

8

LEARNING OBJECTIVES

8.1 Demonstrate an understanding of what plaintiffs must prove if they file a malpractice lawsuit against a counselor.

8.2 Explain what a counselor's duty is if they determine that a counseling client may be at risk of suicide or violence.

8.3 Describe how counselors should proceed differently depending on whether they are faced with a legal issue or with an ethical issue.

8.4 Explain how counselors should react and respond if a complaint is filed against them with their state licensure board.

FOCUS QUESTIONS

1. What do you think are the most common reasons that clients bring malpractice lawsuits against counselors and other mental health professionals?
2. How does a counselor determine whether a client may be at risk of suicide?
3. How do you think you would react if you were a licensed counselor and received a registered letter from your licensure board informing you that you had been accused of professional wrongdoing by a former client?
4. Do you think it is wise to avoid telling friends and colleagues if you receive notification of a complaint against you? Why or why not?
5. In what circumstances do you think you should report another counselor for unethical behavior?

This chapter addresses two potential events that are frightening to counselors: being sued for malpractice and being accused of wrongdoing in a complaint to a counselor licensure board. Although malpractice lawsuits against counselors are rare, they can occur when counselors work with clients who are at risk for suicide or violent behavior toward others. This chapter provides detailed information on steps counselors can take when they are counseling these clients. Counselors also need to understand the process of responding to complaints against them that are made to state counselor licensure boards. It is very uncomfortable for counselors to take action when they suspect a colleague is behaving unethically or illegally, so they should do so only after considering several factors.

The Impact of the COVID-19 Pandemic on Malpractice and Resolving Legal and Ethical Challenges

Few counselors are sued for malpractice, but when they are, the most common complaint is that they failed to manage properly clients who were at risk for suicide. Assessing and managing clients for potential suicide or violence risk has

become more challenging for counselors who provide services through compressed video applications such as Zoom. Fewer visual cues are available to observe when clients are not in the room with the counselor. Also, locating family members or individuals who live near the client and are willing to take responsibility for a client who may be at risk for suicide can be problematic. If counselors must take emergency measures such as calling the police or an ambulance to assist a client who may be at risk for suicide or violence, doing so is much more difficult when counselors live and work at a distance from where clients are located. Counselors who provide counseling services virtually are aware of these issues and are taking steps to ensure they can handle situations in which they are concerned that their clients may be at risk for suicide or violence. Ellington (2022) has offered guidelines for school counselors to consider when they are providing virtual counseling services to students who are exhibiting warning signs of threats to safety.

With the continued expansion of the online education of master's-level counselors-in-training, concerns have been expressed as to whether students who are learning online are adequately prepared to manage clients who might be at risk for self-harm. Elliott and Henninger (2020) measured the impact of three online teaching strategies on students' self-efficacy related to providing counseling services to clients who might be at risk for suicide. They found that all three of the teaching strategies were equally effective, which suggests that online counselors-in-training can benefit from instruction about how to handle clients who could be at risk.

MALPRACTICE

Malpractice involves professional misconduct or unreasonable lack of skill and has been defined as the failure to render professional services to the level expected in a particular community by a prudent member of the profession, with the result that a recipient of services is harmed (Garner, 2014).

Malpractice includes intentional wrongdoing, incompetency, or unintentional wrongdoing of the professional involved. The concept of competency might be extended so that a counselor guilty of *professional misconduct, evil practice*, or *illegal* or *immoral conduct* also could be defined as incompetent.

Malpractice is also a type of civil lawsuit that can be filed against professionals for practicing in a manner that leads to injury to a recipient of their services. Professionals have a legal obligation not to harm individuals who come to them for professional assistance. Although the law cannot restore injured people to their former state of existence, it can require the person who harmed them to compensate them financially for their injuries. If clients believe they have been harmed by their counselors, they can file a malpractice lawsuit against the counselors. Counselors who are sued must then defend themselves against the lawsuit before a judge or jury. For a client plaintiff to prevail in a malpractice lawsuit against a counselor, the plaintiff must prove the following elements (Schwartz et al., 2010):

- The counselor had a duty to the client to use reasonable care in providing counseling services.
- The counselor failed to conform to the required duty of care.
- The client was injured.
- There was a reasonably close causal connection between the conduct of the counselor and the resulting injury (known as *proximate cause*).
- The client suffered an actual loss or was damaged.

Proximate cause is a difficult legal concept to understand. *Actual cause* means that a person actually caused the injury of another person. *Proximate cause* has to do with whether the individual would not have been injured had it not been for the action or inaction of the other person. Just because

professionals are negligent does not make them responsible for an injury. It must be proven that some intervening event (other than the counselor's negligence) did not, in fact, cause the injury. Foreseeability is important in determinations of proximate cause. *Foreseeability* has to do with whether the professional knew or should have known that their actions would result in a specific outcome.

Counselors have become increasingly concerned about being sued for malpractice. For example, because counselors are concerned about whether they should ever touch clients, Calmes et al. (2013) have provided guidelines related to when touching between counselors and clients is appropriate or inappropriate. Although malpractice lawsuits against mental health professionals have increased dramatically in the recent past, the total number of these lawsuits is relatively small. Few malpractice lawsuits are filed against counselors because it is difficult for plaintiffs to establish an adequate case. It is not easy to prove that a counselor deviated from accepted practices and that the counselor's act or negligence caused the harm that a client suffered.

The Healthcare Providers Service Organization (HPSO, 2014) issued a report, covering the 10-year period between 2003 and 2012, that summarized legal actions taken against counselors for whom the HPSO provided professional liability insurance. A second edition, covering the years 2015–2019, was published later (CNA & Healthcare Providers Service Organization [HPSO], 2019). The 2014 report concluded that the most frequent professional liability claims against counselors and the most complaints made to licensure boards involved inappropriate sexual or romantic relationships with clients or the partners or family members of clients. The second- and third-most-common licensure board complaints against counselors were breach of confidentiality and scope of practice issues (e.g., practicing outside the area of a counselor's expertise or authority under licensure laws). Some of the most significant changes from the 2014 report that were published in the 2019 report were: (a) the total incurred costs of claims was $14.7 million, which was double the costs from the earlier report; (b) sexual misconduct and allegations of multiple relationships with clients increased in severity; (c) no telebehavioral health claims were filed through 2019; (d) deposition and record request assistance increased 456%, and child custody matters comprised 37.8% of all assistance; (e) 63.7% of license protection claims closed with no action taken by the board; (f) the average payment of a license protection claim was $5,454, a 46.3% increase; and (g) the highest number of license protection claims were related to complaints of failure to maintain minimal professional standards, sexual misconduct, breach of confidentiality, and reporting to third parties.

The 2014 report, for which data were collected prior to the COVID-19 pandemic, provided other interesting information about claims made against counselors. Almost 67% of complaints were based on face-to-face counseling of an individual client. Even though only about 8% of professional liability claims involved minor clients, payouts from these claims were almost three times higher than payouts related to claims involving adult clients. More than 41% of claims against counselors were made by clients who had sought counseling related to marriage or family discord. Almost 40% of the claims against counselors were based on allegations of an inappropriate sexual or romantic relationship with clients or a spouse, partner, or family member of clients. More than 10% of claims against counselors were accusations that counselors had acted outside their state-defined scope of practice.

According to the 2014 report, one of the biggest payouts to an individual who had sued a counselor was a case in which a male counselor engaged in a sexual relationship with an adult female client while she was in active treatment. The client videotaped a counseling session (without the counselor's knowledge) in which he could clearly be seen striking her. The client filed a complaint with the police, and the police videotaped the counselor stealing from the client's bank account. The claim filed by the client was settled in the $500,000 range.

Although all professional counselors know that it is unethical to have sexual relationships with clients, this case and others demonstrate that some counselors do engage in sex with their clients. For each documented case, there probably are many that are not reported. It is essential that

counselors know themselves and practice self-care so that they do not allow their need for intimate relationships to lead them into sexual relationships with their clients (Keith-Spiegel, 2014).

Insurance companies that provide professional liability insurance for counselors and attorneys who sue and defend counselors in malpractice cases have provided guidelines for preventing being accused of wrongdoing. An insurance company that provides professional liability to counselors, Insureon (2023), has provided advice for preventing malpractice claims:

- Follow standards outlined in the DSM.
- Always maintain strong personal boundaries.
- Avoid inappropriate or excessive self-disclosures.
- Educate yourself on standards of care for counselors that are set by your state licensure board, and make sure you always follow them.
- Make sure your clients have realistic expectations for the services you are providing them, and get an agreement in writing.
- Maintain client confidentiality, but involve family members if clients may be at risk for suicide or violence.
- Keep notes and records that are meticulous, thorough, and accurate.
- Terminate services to clients who attempt to initiate any inappropriate relationship with you, and facilitate their transfer to another counselor.
- Avoid using methods of counseling that are not generally acceptable within the profession.
- Regularly consult with peers.
- Refer clients to specialists when they have issues outside your areas of expertise.

It appears that mental health professionals continue to be sued most often because of sexual relationships with their clients. However, it is likely that the next leading cause of malpractice lawsuits against counselors revolves around situations in which clients attempt or complete suicide (McAdams & Foster, 2000; Roberts et al., 2008). Whether to have sex with clients is certainly a choice that a mental health professional makes, and it is a situation that can be prevented. Predicting whether a client will attempt suicide, however, is scientifically impossible. Yet counselors will be held accountable in courts if they fail to follow procedures endorsed by the profession when a client may be at risk. We provide guidelines for managing clients who may be at risk for suicide and potentially violent clients and discuss the duty to warn intended victims of potential client violence in the following sections.

Clients at Risk for Suicide

When a client threatens to commit suicide, an ethical duty arises to protect the client from harm to self. The ethical standard that applies to clients who pose a danger to others applies to suicidal clients as well, in that confidentiality requirements are waived when disclosure is necessary to protect clients or others from *serious and foreseeable harm* (American Counseling Association [ACA], 2014, *Code of Ethics,* §B.2.a.). Evaluating and managing suicide risk is one of the most stressful situations that you will encounter in your work (Corey et al., 2024; Miller et al., 2013). Suicide prevention programs are an important responsibility for counselors (Gallo, 2017). Cureton and Fink (2019) have suggested that counselors who are assessing client risk factors for suicide also should focus on assessing client protective factors against suicide. Clients who may be at risk for suicide can be assisted when counselors help clients increase the factors that

protect them from making suicide attempts. Suicide prevention measures begin with a thorough risk assessment and then, depending on the level of danger, might include involving the client's family or significant others, working with the client to arrange for voluntary hospitalization, or even initiating the process that leads to an involuntary commitment of the client. All of these interventions are disruptive and compromise the client's confidentiality. Ethically (ACA, 2014, §B.2.e.) and legally under the Health Insurance Portability and Accounting Act (HIPAA), it is important to disclose only the information that you consider essential for someone else to help prevent a suicide attempt.

To take action to protect a client who may be at risk for self-harm, the counselor must first make the decision that the client may be at risk. In a qualitative study of suicide attempt survivors, Sheehan et al. (2021) found that the survivors decided to disclose their intended suicide attempts based on individualized circumstances and the context of the survivors' lives. The results of this study support the concept that decisions to attempt to harm oneself are individualized and do not follow identifiable patterns. Even though there is no sure way to predict whether a particular client will attempt suicide, experts agree that individuals who die by suicide generally give clues beforehand to those around them (Barbour et al., 2021; Capuzzi, 2002; Sallee et al., 2022). Day-Vines (2007) has alerted counselors to the soaring rates of suicide among African Americans in the United States. In a qualitative study of active-duty military spouses, Cole et al. (2021) found that the participants had a fear of seeking mental health services and had difficulty accessing such services, putting that population more at risk for suicide. Information such as this must guide the day-to-day work of counselors when they assess clients for suicide potential.

In most circumstances, a counselor's determination of a client's level of risk must be based on clinical observations, not on test results. If counselors were not prepared in their graduate programs to handle crises involving the possibility of client suicide (Allen et al., 2002), they must remediate this deficiency through independent reading, workshop attendance, post-master's-degree course completion, and supervised practice (McGlothlin et al., 2005; Morris & Minton, 2012; Shannonhouse et al., 2017). Results of studies have shown that preparing counselors in training to intervene when clients may be at risk for suicide is effective (Griesbach et al., 2008; Shannonhouse et al., 2018).

As noted, determining that a client may be at risk of suicide leads to counselor actions that can be exceptionally disruptive to the client's life. Just as counselors can be accused of malpractice for neglecting to take action to prevent harm when a client is determined to be at risk for suicide, counselors also can be accused of wrongdoing if they overreact and precipitously take actions that violate a client's privacy or freedom when there is no basis for doing so (Hawton et al., 2022; Hermann et al., 2011). As a result, counselors have a legal duty to evaluate accurately a client's potential for suicide. Counselors can be held liable for overreacting and for underreacting. So, how should a determination be made as to whether a client is suicidal?

First, no matter where you work as a counselor, you are likely to provide services to individuals who might express suicidal thoughts (Hermann, 2001; O'Dwyer, 2012). Therefore, it is necessary for all counselors to know the warning signs that a particular person is at risk for suicide. Because the act of suicide is done by the client without the counselor being a party to it, courts generally have been reluctant to hold counselors accountable for harm that results from clients who attempt or complete suicide.

A legal case from the 1960s (*Bogust v. Iverson*, 1960) held that a college counselor was not a mental health professional and therefore had no duty to assess a client's risk of suicide. Since then, however, counselors have established themselves as mental health professionals, and the law imposes on counselors—practicing in all settings—the responsibility of knowing how to accurately determine a client's risk of suicide (*Eisel v. Board of Education of Montgomery County*, 1991). Some school counselors may be reluctant to assess students for suicide (Becnel et al., 2021), thinking they can prevent their liability by neglecting to make such assessments, but Gallo and Wachter Morris (2022)

and Wachter Morris and Gallo (2022) have argued that school counselors are properly prepared to conduct suicide assessments and to provide suicide prevention programs in schools and that school counselors should not avoid working with children who may be considering suicide or choose not to perform suicide assessments due to fear of liability. Morris et al. (2021) have provided a suggested process for school counselors to determine how to measure the risk of potential student suicide among their particular school's student population and how to structure a reasonable suicide prevention program. Gibson (2022) has pointed out that even though school counseling began with career counseling and guidance, it has evolved to a point where school counselors are now being prepared to address the growing mental health needs of school-aged children.

Much help is available in the professional literature, including research studies and articles that provide information about warning signs of future suicidal behavior (Daniel & Goldston, 2012; Sallee et al., 2021; Sapyta et al., 2012). Today's counselors must know how to make assessments of a client's risk for suicide and must be able to defend their decisions at a later time (Linehan et al., 2012). Binkley and Elliott (2021) found published articles about how counselors are taught to respond to potentially suicidal clients, but the literature is lacking in articles about the process used to prepare counselors to identify clients who may be at risk for suicide and how to respond once at-risk clients are identified.

In recent years, the professional counseling literature has introduced considerable discussion regarding self-injury, and the term *nonsuicidal self-injury* is often used, which might be a bit misleading. Ross and Heath (2002) defined "nonsuicidal self-injury" as behaviors that involve intentional bodily harm that causes tissue damage without an individual having the intent to die, and the International Society for the Study of Self-Injury (2023) added to that definition "and for purposes not socially or culturally sanctioned." Self-injury includes activities such as cutting, burning, self-bruising, breaking bones (International Society of the Study of Self-Injury, 2023; Zetterqvist, 2015), carving of the skin, subdermal tissue scratching, banging or punching objects or oneself, embedding objects under the skin (Self-Injury and Recovery Research and Resources, 2023), and excoriation, also known as *skin picking* (Jagger & Sterner, 2016). The literature has explored the question of whether self-injury is associated with suicide (Whisenhunt et al., 2014), and contains suggestions for providing counseling to clients who are engaged in self-injury (Lawrence, 2023; Nelson & Piccin, 2016; Rutt et al., 2016; Stargell et al., 2017, 2018), developing nonsuicidal self-injury prevention programs (Stargell et al., 2017, 2018; Wester et al., 2017), and locating detailed information regarding various forms of self-injury (Jagger & Sterner, 2016).

Best practices in counseling individuals who self-injure have been summarized by Calvo et al. (2022) and Turner et al. (2014), but Fox et al., (2020) have pointed out that the efficacy of such treatments is still unclear. The goal of counseling with persons who self-injure is to reduce nonsuicidal self-injury by focusing on the thoughts, emotions, and behaviors that cause a person to self-injure (International Society of the Study of Self-Injury, 2023).

The prevailing belief among mental health professionals is that self-injury is not automatically a predictor of suicide attempts (Griep & MacKinnon, 2020; Kiekens et al., 2018; Ribeiro et al., 2015; Walsh, 2006), although certainly some people who self-injure might also be suicidal, and the risk of suicide is heightened among clients who self-injure (Brausch & Gutierrez, 2010; Toprak et al., 2011). Kakhnovets et al. (2010) found that the rate of suicidal ideation increased from 31.8% for those who reported one episode of self-injury to 60% for those who reported more than one episode of self-injury. Wester et al. (2016), after a review of the literature, reported that 10% to 37% of those who engaged in nonsuicidal self-injury also attempted suicide at some point in their lives. Nock et al. (2007) and Nock et al. (2010) found that suicidal thoughts generally do not accompany self-injurious behavior. It seems that self-injury and suicidal ideation are two separate behaviors and therefore should be assessed separately (Wester & McKibben, 2016).

8-1 The Case of Monica

Monica is a new counselor at a community mental health center. She has taken this position as her first job after completing her master's degree in counseling. Ashley is a 15-year-old client with whom Monica has had two counseling sessions. Ashley's mother drives her to counseling sessions and waits for her until the sessions are over and then drives her home. Ashley told Monica in their first session that she sometimes cuts herself with small incisions above her ankle and below her knee. Ashley assured Monica that the cuts were never deep, rarely produced any blood, and were not in any way dangerous. In that first session, Monica talked with Ashley about self-injury and what is known about such behaviors. She asked Ashley questions about what precipitates her cutting behaviors; what her feelings are before, during, and after cutting; and what her thoughts were about cutting. In the second session, Ashley revealed to Monica that she worries that some day she might cut herself too deeply and might start bleeding too much. Ashley begs Monica not to tell her mother what she has told her.

- What factors should Monica consider when determining how to respond to Ashley's request not to tell her mother?
- How do you think Ashley will respond if Monica tells Ashley's mother about Ashley's cutting behavior over Ashley's objection?
- What are the consequences if Monica does not tell Ashley's mother about Ashley's cutting behavior and Ashley later cuts and injures herself seriously or attempts or completes suicide?
- In this situation, given these circumstances, do you think Monica should tell Ashley's mother about the cutting behavior?

Discussion: First, Monica should ask Ashley to give her more information about the cutting behaviors. Monica should ask Ashley if she actually has cut herself so deeply that she has bled substantially. Monica needs to determine whether Ashley is fearful of cutting herself too deeply in the future, or if perhaps she has already done that in the past, and if she has cut herself deeply in the past, how many times. If Monica determines that Ashley is involved only in surface cutting and has not put herself in danger but is only concerned about perhaps doing that in the future, Monica could determine that Ashley is not currently a danger to herself and could continue her counseling sessions with Ashley. Monica would want to closely monitor Ashley's cutting behaviors from week to week. If Monica makes the determination that Ashley is not currently a danger to herself, she should consult with her supervisor at the agency, give her supervisor complete information about Ashley, and then follow the directives of her supervisor regarding how to proceed in her counseling sessions with Ashley. If, after questioning Ashley thoroughly about her cutting behavior, Monica determines that Ashley might be engaged in cutting behaviors that might constitute a danger to herself, then Monica should consult with her supervisor immediately. Monica may decide to bring Ashley's mother into the session. Monica can either convince Ashley to tell her mother or tell Ashley's mother herself what Ashley has told her. Monica should advise Ashley's mother to have Ashley evaluated by a physician to determine if her cutting behaviors constitute a danger to her health.

After reviewing the literature, Whisenhunt et al. (2014) concluded that common risk factors for both suicide and self-injury include substance misuse, low self-esteem, suicidal ideation, disordered eating, and non-heterosexual identity. But self-injury risk factors include factors that are not predictive of suicide, including history of abuse or current abuse, eating

disorders, early first sexual encounter, and low socio-economic status (Whisenhunt et al., 2014). And suicide risk factors that do not predict self-injury include broken homes, mental illness in self or family members, female gender, risky weight control behaviors, early onset puberty, recent suicide attempts by a friend, and behavioral problems. Wester and McKibben (2016) explored the experiences of 88 individuals who had engaged in self-injury and concluded that there was evidence that self-injury can desensitize individuals to commit suicide, although 38% of the participants indicated that nonsuicidal self-injury delayed or protected them from attempting suicide.

It is possible that clients who engage in nonsuicidal self-injury may unintentionally harm themselves severely. Buser et al. (2017) concluded that individuals who self-injure and have addictive qualities (which include tolerance of the pain, time-consuming nature of self-injury, and continuation despite negative consequences) are more at risk for unintentional severe harm than those who self-injure but who do not have addictive qualities.

As a result of a qualitative study in which eight licensed professional counselors who were experienced in working with clients who self-injure were interviewed, Whisenhunt et al. (2014) concluded that there was a lack of consensus among the participants about the relationship between self-injury and suicide. These researchers believed that this finding reflected the continued professional debate surrounding the topic.

The association between self-injury and suicidal ideation remains unclear. As a result, counselors who treat those who self-injure should always assess for suicidal ideation (Wester et al., 2016). Instruments are available to assist counselors in assessing clients for self-injurious behavior (Buser & Peterson, 2016) and in assessing clients for risk of suicide (Erickson & Abel, 2013; Lotito & Cook, 2015; McGlothlin et al., 2016).

HOW BULLYING/CYBERBULLYING CAN LEAD TO SUICIDE Bullying has been associated with suicide in that some victims of bullying do attempt or die by suicide. The Pacer National Bullying Prevention Center (2023) has defined bullying as occurring when a person aggressively uses their power to target another individual with repeated, unwanted words or actions. Bullying can occur in person or online. Bullies have more social or physical power than their victims. Bullying can be repeated or can be a one-time incident. Bullying can also occur outside of schools, and adults sometimes accuse other adults of bullying.

Professionals who counsel clients who are either victims of bullies or perpetrators of bullying should be aware that victims sometimes attempt or complete suicide. Therefore, counselors should assess bullying victims for suicidal ideation. To help prevent suicide, counselors should consider starting bullying prevention programs.

The Pacer National Bullying Prevention Center (2023) has indicated that cyberbullying takes place over digital devices such as cell phones, tablets, and computers and can occur using email, texting, social media, gaming, instant messaging, or photo sharing. The Centers for Disease Control and Prevention (2023) has estimated that 15.9% of high school students in the United States were electronically bullied in the year 2021. Suggestions for preventing cyberbullying and for counseling those targeted by cyberbullying have been developed by several scholars (Burnham et al., 2011; Hinduja & Patchin, 2019; McCormac, 2014). The Pacer National Bullying Center (2023) offers specific suggestions for helping youth who have experienced cyberbullying that include talking with children about the potential for cyberbullying, giving children specific advice about electronic communications, and providing children with steps to take when they report having experienced cyberbullying.

MAKING ASSESSMENTS FOR SUICIDE RISK The law does not require that counselors always be correct in making their assessments of suicide risk, but it is a legal requirement that counselors make those assessments from an informed position and that they fulfill their professional obligations to a client in a manner comparable to what other reasonable counselors operating in a similar situation would do. Because of this standard of care to which counselors are held, the very best action you can take if you are unsure whether a client is at risk for a suicide attempt is to consult with other mental health professionals who are similar to you. The *Code of Ethics* (ACA, 2014, §C.2.e.) advises counselors to consult with other counselors when they have questions regarding their ethical obligations or professional practice. It also is important to look for consensus among your consultants and certainly to follow their advice in making your final decision about what to do in a particular case. In situations in which you have assessed a client's potential risk for suicide, it is essential that you document the following items carefully, regardless of whether you determine that the client may be or is not currently at risk. Essential items to include in your documentation notes include:

- What precipitated your concern about the client (such as referral by another person, or something the client said or did)
- Questions you asked the client and their responses
- Individuals you consulted regarding the situation, what you said to them, and how they responded
- Interactions you had with any other persons regarding the situation, from the time you became concerned until you completed your work for the time being regarding the situation.

When you make the decision that a client may be a danger to self, you must take whatever steps are necessary to prevent the harm, and your actions must be the least intrusive to accomplish that result. Again, consulting with colleagues could be very helpful. Many counselors who have determined that a client may be at risk for suicide require that the client submit to an evaluation by a mental health professional who has expertise in suicide, as a condition for continuing to receive counseling services from that counselor. For example, you might require that your client see a psychiatrist on the staff at the facility where you work, if that is an option. Or you might require your client to submit to a mental evaluation at a local hospital where psychiatric services are available. Less intrusive options are also available, such as referring the client to a primary care physician if the client is in a health plan that requires that step before gaining access to a specialist, such as a psychiatrist. Of course, you should choose a less intrusive option only if you are sure the client is not at imminent risk.

The Centers for Disease Control and Prevention (2023) maintains a website related to suicide prevention that is continually updated with the latest research reports. Because it is so difficult to decide what steps to take in a crisis situation, especially one in which a client's life may be in danger, we have provided an action plan to follow if you determine that an adult client may be at risk for suicide (see Figure 8-1). If your client is a minor, you must always notify the parents or guardians (Capuzzi, 2002). Responsibilities to minors and their parents or guardians are discussed further in Chapter 11. The steps we suggest are not the only options that counselors have. Rather, we are providing one possible way to manage potentially suicidal clients that hopefully will yield positive results.

If you work in an agency, school, hospital, or other organizational setting, you must always follow the procedures established by your employer. In many cases, counselors find that their employer does not have guidelines for how to manage potentially suicidal clients. If that is the case, you should consider following the guidelines in Figure 8-1 until such policies are adopted. In addition, you should ask that guidelines be established for the welfare of clients and to protect the

FIGURE 8-1 Steps to follow if you determine that your client may be at risk for suicide

If you determine that an adult client has exhibited some behaviors that are related to suicide, but currently does not appear to be at risk for suicide:

1. Summarize in your case notes the client's behavior that supports your concern. Do not write that you believe the client may be at risk for suicide. Instead, write that although you do not believe the client may be at risk for suicide, you performed an assessment. Give details about how you assessed the client. You could decide that the client is fine and that you do not have to tell anyone about what the client said or did that led to your concerns. If you believe a significant person in their life needs to be informed of the behaviors that concern you, then document that determination in your notes.
2. Consultation is recommended. If you have consulted with colleagues, experts, or supervisors in reaching your position, document the consultations in your case notes.
3. Tell your client your concerns and, if appropriate, obtain an agreement from the client to inform a significant person in their life of your concerns. Tell the client to have that person contact you as soon as possible.
4. If your client is not capable of telling the significant person or if, for some other reason, asking the client to inform the significant person does not seem like an appropriate course of action, explain to the client that you will be contacting the significant person to share your concerns. If you are not in independent private practice, inform your supervisor of the actions you will be taking and follow any directives given.
5. Choose a significant person in your client's life (such as your client's spouse or partner, parent, adult child, other relative, or close friend), and inform them of the situation. Choose a responsible person who lives with the client or who is in frequent contact with the client.
6. Document in your case notes all conversations with your client, the client's significant person, and your supervisors.

If you determine that an adult client MAY BE at risk for suicide . . .

1. You are dealing with a very serious matter that requires immediate and decisive action. Make the determination that an adult client may be at risk for suicide only if the client has made a suicide gesture or attempt, has told you or someone else in a believable fashion that they plan to attempt suicide, or has engaged in a pattern of behavior that the professional literature suggests is characteristic of an adult who is at risk for suicide. Follow any agency policies that exist regarding managing adults at risk for suicide. If you are not in an independent private practice, notify your supervisor of the situation, and follow any directives given. If agency policies dictate or if your supervisor directs you to proceed differently from the following steps, follow the policies or the orders of your supervisor.
2. Consultation is recommended. If you have consulted with colleagues, experts, or supervisors in reaching your position, document the consultations in your case notes.
3. Explain to your client that you will have to notify a significant person in the client's life so that the person can help.
4. Assure your client that you will continue to help and that you will disclose only the minimum information necessary to get assistance for the client. Try to calm the client, but do not minimize the seriousness of the situation. Explain what may happen in the next few hours, next few days, and long term.
5. Ensure that your client is not left alone and does not have any opportunity for self-harm before turning the client over to the significant person.
6. Contact the significant person in your client's life, and explain that you believe their relative, partner, or friend may be at risk for suicide. Give specific details that led to your concern. Insist that the significant person come to pick up the client immediately.
7. If a significant person cannot be found, make sure your client is under the supervision of a responsible person until a significant person is located.
8. If you cannot contact a significant person and if it is impossible to keep your adult client safe for an extended period of time, call an ambulance and have the client transported to a hospital that has psychiatric services. If you are not in an independent private practice, be sure to inform your supervisor and obtain permission and support before taking this action. If your supervisor directs you to take a different course of action, do so and document in your case notes what you were told and did.

FIGURE 8-1 (*continued*)

Give the ambulance attendant your contact information and offer to speak with the person at the hospital who will be conducting the evaluation, if requested to do so. Continue to attempt to contact the client's significant person.

9. When you talk to the significant person, ask that person to immediately take possession of and responsibility for your client.
10. When the significant person arrives, explain that you believe your client may be at risk for suicide, give specific details that led to your concern, instruct the significant person what to do next, and ask that a document be signed that acknowledges that the significant person has been informed of your concerns, has been given directions regarding the steps to take next, and has agreed to take responsibility for your client.
11. Have the client sign a form giving you permission to disclose any information you have to mental health professionals who may evaluate or treat the client in the future. If the client refuses or is not capable of signing the form, ask the significant person to sign on your client's behalf.
12. Explain to the significant person that they must ensure that your client is not left alone, does not have any opportunities for self-harm, and is taken for an evaluation as soon as possible to determine whether they are at risk for suicide.
13. If the significant person refuses to sign the document or communicates to you in some way that they will not take the situation seriously, call an ambulance, and follow the steps in item 8.
14. As events occur, detail in your case notes all the events that transpired in relation to this situation. Be sure to date each entry and indicate the time you wrote it. Make several entries if necessary, and do not delay in writing details in your case notes.
15. When your client returns to you or your agency for services, obtain written permission from your client to contact the professional who determined that your client was not at risk for suicide or was no longer at risk for suicide.
16. Contact your client's treating physician, psychologist, or other mental health provider, and explain that the client has returned to you or your agency for services. Ask the treating provider to summarize their evaluation and treatment of the client. Inquire as to whether the provider will continue to treat the client and, if so, the details of the planned treatment. Also ask the treating provider the types of counseling services they would like you to provide to the client. Do not agree to provide any counseling services that your position does not allow you to provide. Ask the provider to tell you the circumstances in which you should return the client to them for further evaluation or treatment.
17. As soon as possible after you have talked to the provider, document in your case notes the details of your conversation. Be sure to date the entry and indicate the time you wrote it.

counselors who have to make decisions in difficult situations. If your administrative supervisor is present when you are involved with a potentially suicidal client, you must follow your supervisor's directives. The guidelines presented in Figure 8-1 could be adopted by your agency, or you could follow these in the absence of any agency policies or directives from superiors.

As you can see in Figure 8-1, it is assumed that a client you refer for an evaluation will return to you for services. Even if a client is hospitalized, the hospitalization usually is only a few days in duration. Contacting and documenting your consultation with the mental health professional who determined that your client was not at risk for suicide, or was no longer at risk, is vital. You probably will counsel clients who have recently been at risk for suicide. In an interesting study, Paulson and Worth (2002) found that clients who previously had been at risk for suicide described these key therapeutic processes that helped them to overcome suicidal ideation and behaviors: (a) experiencing an affirming and validating relationship as a means of reconnection with others, (b) dealing with the intense emotions surrounding suicidal behavior, and (c) confronting and discarding negative patterns while establishing new, more positive behaviors.

Clients Who May Be at Risk for Harming Others

Counselors are often responsible for violence prevention education (Banyard, 2022; Carter et al., 2022; D'Andrea, 2004; Duarte & Hatch, 2014; Ginter, 2004; Smith & Sandhu, 2004). In addition, there will be situations during your counseling career when you must decide whether a particular client has the potential of harming another person or perhaps even an individual's property (Chambers et al., 2010; Höffler et al., 2022; McAdams et al., 2011; Radatz & Zoe Hilton, 2022; Winer & Halgin, 2016). Making the decision as to whether a client may be violent in the future is difficult, as there is no scientific certainty for such a decision. If you do determine that a client may be a danger to another person, then you must take the steps necessary to prevent harm to that other person (Gilbert, 2007; Hermann & Finn, 2002). Steps may include warning intended victims by contacting them directly or notifying authorities of the threat. In making these difficult decisions, it is essential to consult with other mental health professionals and to include supervisors to the extent possible.

What began as a legal requirement has now evolved into an ethical duty as well. Standard B.2.a. of the *Code of Ethics* (ACA, 2014) states that the counselor's confidentiality requirement "does not apply when disclosure is required to protect clients or identified others from serious and foreseeable harm." This particular exception to confidentiality has caused considerable confusion and consternation among helping professionals, not only because it involves breaching confidentiality but also because it demands that counselors be able to predict dangerousness. Human behavior is not always predictable, and counselors may find themselves in a dilemma, both ethically and legally, in determining whether to breach a client's confidentiality to prevent harm to the client or to others.

8-2 The Case of Javier

Javier is a counselor in a community mental health center. For the last two weeks he has been seeing a client named Bill, who is a junior in high school. Bill comes to the center one day each week, walking from school to his sessions and then walking home afterward. This afternoon, Bill tells Javier that, after thinking about it for quite some time, he has decided to shoot the assistant principal at his school. Bill says that his father has a rifle collection in their home, and he has access to the guns and to ammunition. Bill explains that he plans to wait outside the assistant principal's home until he walks out in the morning and to shoot him as he walks to his car. Because Bill is so calm as he relates all of this information, Javier asks Bill if he is serious. Bill smiles, laughs, and then says, "No, I was only kidding. I would never do anything like that." Because Javier doesn't know Bill very well, he is not sure what to do next. Javier asks Bill to excuse him for a minute and leaves his office to look for his supervisor. He discovers that his supervisor and all the other staff have left for the day. Javier returns to his office and finds Bill standing by a window, looking out with tears in his eyes.

- In these circumstances, what are Javier's options?
- What option would you choose if you were Javier?

Discussion: Javier has a number of options in this situation. He could (a) talk more with Bill and try to assess his potential for harming the assistant principal, (b) call Bill's mother or father to come to the center to pick Bill up, and tell the parents what Bill told him, (c) call the police and ask them to come over to talk with Bill, or (d) call the assistant principal and warn them of the threat against his life.

Javier's best option might be to talk further with Bill. If Javier determines that Bill was not serious and therefore is not a danger to the assistant principal, Javier should document what happened in his notes and explain why he reached the conclusion he reached. Even if Javier determines that Bill might not have been serious, Javier should call Bill's parents and ask them to come over. He should get them to take responsibility for Bill's actions and the assistant principal's safety.

If Javier determines that Bill may be at risk for harming others, in this case the assistant principal, then Jaiver must take steps to prevent the harm. If the parents are available, Javier could have one or both come to the center, explain what Bill told him, and ask them to take responsibility for having Bill evaluated by a professional who could admit him to a hospital if necessary to determine whether Bill is a danger to others. In this case, unless Javier is practicing in Texas, Javier should inform the assistant principal about what Bill said, and if Javier cannot reach the assistant principal, Javier should call law authorities and tell them what Bill told him and ask them to warn and protect the assistant principal.

Research indicates that it is impossible to predict whether a particular person is going to harm someone else; yet the law and our ethical standards require counselors to determine whether a client may be dangerous to others. How can this legal and ethical duty be fulfilled? First, it is important to learn as much as you can about the warning signs of persons who commit violent acts against others (Daniels, 2002; Haggard-Grann, 2007; Höffler et al., 2022; Radatz & Zoe Hilton, 2022; Truscott & Evans, 2001; Whiting et al., 2020). For example, it appears that the best predictor of violence is past violent behavior. However, you must be careful not to profile, or to assume that a category of persons is prone to violence (Bailey, 2001). Special situations, such as those involving intimate partner violence (Lawson, 2001, 2003) or violence committed by persons with serious mental disorders, probably require deviations from the general guidelines offered in this chapter.

Once you have determined that a client may indeed be dangerous and might harm another person, the law requires that you take whatever steps are necessary to prevent the harm and, further, that the steps you take be the least disruptive. You have choices that range from the least intrusive action (obtaining a promise from your client not to harm anyone else) to the most intrusive (having the client involuntarily committed to a psychiatric facility). Numerous possibilities exist between these two extremes, such as notifying the client's family members and getting one of them to take responsibility for keeping the client under control, persuading the client to be voluntarily committed to a residential facility, notifying the police, or calling the client periodically. Certainly, no particular formula of action is necessarily correct when you determine that your client may be dangerous. If at all possible, consult with other mental health professionals, use their input in making your decision, and be sure to document such consultations in your notes.

The guidelines in Figure 8-1 regarding managing potentially suicidal clients would work equally well in managing potentially violent clients. You would just need to add the determination of whether to warn intended victims, law enforcement authorities, or both.

DUTY TO WARN INTENDED VICTIMS Think about how you might react if you were counseling a client who made a threat to harm someone else. If you were to decide that the client was only venting anger and was not likely to act on the threat and then later, the client carried out the threat, you would have acted ethically in trusting your professional judgment and in preserving confidentiality and the counseling relationship. That would be little solace, however, if someone were killed or seriously injured. Also, the question might arise before an ethics committee or in a court as to whether you could have acted, or should have acted, to avert that harm. On the other hand, if you were to

decide that the danger might be in fact clear and imminent and you were to issue a warning, no harm would be done to a third party. If the client did not later attempt to harm the possible victim, however, you might have erred in labeling the client dangerous, and your actions probably will have destroyed the counseling relationship. You could be accused of misdiagnosis, defamation of character, or violating confidentiality (*Hopewell v. Adebimpe*, 1982). It is no wonder that counselors view these as no-win situations and might be tempted to simply avoid working with dangerous clients. Yet, it would be ethically questionable to close the doors of the counseling profession to a group of individuals who clearly need our help. A client in any setting might become violent and dangerous.

The duty to warn an identifiable or foreseeable victim of a "dangerous client" arose out of the landmark *Tarasoff v. Regents of University of California* (1976) court case, which established the legal concept in California that if a counselor determines or should have determined that a client presents a serious danger of violence to others, then the counselor must take reasonable steps to protect the foreseeable victim from danger.

Although the *Tarasoff* doctrine has not been accepted or applied in every jurisdiction in the United States, counselors have chosen to incorporate it into the *Code of Ethics* (ACA, 2014) and generally assume that the concept is a national legal requirement. The only jurisdiction that has specifically rejected the *Tarasoff* duty-to-warn doctrine is Texas (*Thapar v. Zezulka*, 1999). In Texas, counselors do not have a duty to warn intended victims if their clients threaten to harm another person. Additionally, in Texas, a counselor may issue a warning but is not required to do so, and the intended victim is not among those parties who can be warned. As a result, if a mental health professional in Texas did warn an intended victim, the professional might potentially be liable to the client who made the threat for breach of confidentiality.

The facts in the *Tarasoff* case are important in understanding the duty that has been imposed on counselors. Prosenjit Poddar was a 26-year-old graduate student at the University of California at Berkeley. In the course of a session with his counselor, Poddar confided his intention to kill his girlfriend, Tatiana Tarasoff. His counselor was Dr. Lawrence Moore, a psychologist in the university counseling center. Because Moore believed that Poddar might be serious regarding his threat, Moore initiated proceedings to have Poddar committed for a psychiatric evaluation: Moore orally notified two campus police officers of his intentions to commit Poddar and then sent a letter to the police chief requesting assistance from the police department. The police took Poddar into custody but released him when he promised not to contact Tarasoff. Poddar never went to see Moore again. Neither Moore nor the police notified Tarasoff of Poddar's threat. Two months later, Poddar stabbed and killed Tarasoff on the front porch of her parents' home. Tarasoff's parents sued Moore in a wrongful death action for not confining Poddar and not warning Tarasoff of the threat against her life.

Decisions subsequent to the *Tarasoff* case throughout the United States have interpreted the holding of the case in different ways. Many courts have limited its application to situations in which the victims are readily identifiable. Other courts, however, have extended the duty to all persons who are endangered by a patient's condition or threats. In *Lipari v. Sears, Roebuck & Co.* (1987), a federal court held that a psychotherapist has a duty to warn and protect unknown victims, as well as those who are readily identifiable. This case extended a counselor's duty to those persons who are foreseeably endangered by a client's conduct.

A Washington state court, following the *Lipari* decision, held that a psychiatrist could possibly be held responsible for injuries sustained by a traffic accident victim. The victim was hurt when a drug-abusing patient, released by the psychiatrist, ran a red light while under the influence of drugs (*Petersen v. State,*1983). Two California cases expanded the *Tarasoff* doctrine by no longer requiring that a psychotherapist be able to readily identify the patient's victim (*Hedlund v. Superior Court of Orange County,* 1983; *Jablonski v. United States,* 1983).

In a Vermont case, the state supreme court held that a mental health professional has a duty to take reasonable steps to protect a third party from property damage after threats are posed by a patient (*Peck v. Counseling Service of Addison County, Inc.*, 1985). In the *Peck* case, the court found that a counselor could be held responsible for property damage when the client burned down a barn. The client had told the counselor he intended to do so, but the counselor did not warn the owners of the barn.

Originally, the *Tarasoff* holding imposed a duty to warn only if the victim was specifically identifiable. Subsequent decisions have extended that duty to include warning persons who are unknown, persons who are unintentionally injured by a patient, whole classes of persons of which the victim is a member, bystanders who might be injured by a patient's negligent act, and individuals whose property a client has threatened to destroy. Fox (2010) has suggested, however, that courts now are tending to limit a mental health practitioner's duty to protect third parties from potentially dangerous clients.

Pabian et al. (2009) found that 76.4% of the psychologists in four states were misinformed about their state laws regarding *Tarasoff*-type situations and believed that they had a legal duty to warn intended victims even though they did not have that duty. Decision making for counselors around the *Tarasoff* legal requirements is complex. One thing is clear, however: Because of the *Tarasoff* case, when you determine that a client might harm an identifiable or foreseeable person, you must directly or indirectly warn that individual of the danger, except in Texas, where you may warn law enforcement personnel. It is important that you know what the statute and case law holds in the state where you practice regarding your duty to warn intended victims.

CLIENTS WITH HIV According to the ACA (2014) *Code of Ethics*' *Contagious, Life-Threatening Diseases* (§B.2.b.), "When clients disclose they have a disease commonly known to be both communicable and life threatening, counselors may be justified in disclosing information to identifiable third parties, if the parties are known to be at serious and foreseeable risk of contracting the disease." This standard goes on to say that before making a disclosure of this type, counselors should "assess the intent of clients to inform the third parties about their disease or to engage in any behaviors that may be harmful to an identifiable third party." In addition, "Counselors [should] adhere to relevant state laws concerning disclosure about disease status."

Joe (2018) has pointed out that one problematic aspect of this standard, §B.2.b., is that what is commonly known about HIV may make the standard outdated and incorrect. Joe suggested that counselors might serve clients better by relying on their ethical obligation to seek continuing education (§C.2.f.) to stay current about HIV prevention, transmission, and treatment.

The ACA *Code of Ethics* (2014) section that is related to "communicable and life-threatening" diseases and that allows counselors to disclose the client's having such a disease to third persons "… if the parties are known to be at serious and foreseeable risk of contracting the disease" was adopted when much less was known about HIV, which was the primary disease known at that time that could be transmitted from one person to another and was life-threatening.

It is important to note that, according to ethical guidelines, counselors are *justified* in disclosing, but are not necessarily *required* to disclose, information to a third party who may be *at serious and foreseeable risk*. Due to medical advances, HIV and AIDS are no longer routinely communicable or fatal, which makes the applicability of this standard to HIV/AIDS open to question. According to the Centers for Disease Control and Prevention (2023), although there currently is no cure for HIV, it can be controlled, and with effective treatment, those living with HIV can live long, healthy lives and protect their partners.

An estimated 1.2 million people in the United States had HIV at the end of 2019 (Centers for Disease Control and Prevention, 2023). In 2020, 30,635 people received an HIV diagnosis in the United States. It is important for counselors who provide services to persons with HIV to inform

themselves about the disease and to utilize evidence-based approaches for counseling that population (Marc et al., 2022).

Your work with clients who may be dangerous to themselves or others will be fraught with ethical and legal complexities. It is thus important to keep up with the literature in this area and to familiarize yourself with guidelines for practice regarding the duty to warn. Whenever possible, you should consult with fellow professionals (ACA, 2014, §C.2.e.) if you are in doubt about whether you have a duty to warn, protect, or report in a given situation.

A Hypothetical Malpractice Case

Counselors might be sued for malpractice any time clients believe they have been harmed because of actions counselors took or failed to take. To illustrate how a malpractice case might be considered against a counselor, we analyze a plausible situation in which a counselor might be sued. Suppose that a teacher has come to Himari, a high school counselor, with a concern about Mark, an 11th-grade student. Mark has written a paper for English class in which he discusses taking his life because he is upset about his parents' recent divorce. Himari calls Mark into her office and talks with him about the paper. She asks him if he has seriously considered taking his life. After a thorough discussion with Mark, Himari determines that he is not suicidal.

That same day, after school, Mark shoots and kills himself with his father's rifle. Mark's mother, Sheila, finds out about the teacher's referral and files a malpractice lawsuit against Himari, claiming that Himari's incompetence caused her son's death. Sheila's attorney probably would name as respondents in the lawsuit Himari individually, each of her direct supervisors up through the school superintendent, the school board, and any other individuals or groups who might be responsible for Himari's actions or failure to act. If Himari had her own independent professional liability insurance policy, her insurance company would hire a lawyer who would represent Himari individually in the case. If Himari did not have her own liability insurance policy, she probably could not afford to pay an attorney to represent her. The average cost of a lawsuit, including legal fees and judgments awarded, is $113,642 (Healthcare Providers Service Organization [HPSO], 2022). Himari would have to rely for her defense on the school system's lawyers, who ultimately represent the school system, not her. The school system would have its lawyers, or the lawyers provided by the school system's liability insurance company, file a legal response to the case. Himari's lawyer, if she had one, and the school system lawyers would work together in defending the lawsuit.

Sheila's attorney, in presenting her case in court, would first have to prove that Himari owed a duty of reasonable care in providing counseling services to Mark. This would not be difficult to prove because Himari is a counselor in the school and called Mark in for a counseling session. When a counselor accepts a person as a client, the counselor then has a duty to provide the client with professional care that meets accepted standards within the profession. Counselors have a fiduciary relationship with clients, which is a relationship that fosters the highest level of trust and confidence (Anderson & Bertram, 2012). Clearly, Himari owed Mark a duty of care.

After establishing that a duty of care was owed to Mark, Sheila's attorney then would have to prove that Himari breached that duty and failed to conform to the required standard of care. Most experts would agree that counselors must assess whether a client is at risk for suicide, even though studies have shown that it is impossible to scientifically predict suicidal behavior (Hawton et al., 2022). Large (2018) has said that categorizing groups that might be more at risk for suicide results in too many errors in deciding whether a person is at risk for suicide, misses many who do complete suicide, and therefore is not helpful in preventing suicide. Large's position was that the assessment of clients for suicide risk should focus on the current situation and needs of the client rather than on probabilistic notions of suicide risk.

Despite the impossibility of predicting suicide, counselors still must determine whether an individual may be at risk for suicide. If it is determined that the client may be in danger, action must be taken to prevent the possible impending suicide. The question in this hypothetical case is whether Himari was reasonable in her assessment of Mark's risk. The only way to prove that Himari made an error in her professional judgment is for Sheila's attorney to bring in one or more expert witnesses who will testify that a competent counselor would not have done what Himari did or failed to do. The expert witnesses must compare the actions Himari took or did not take to the actions that a reasonable counselor with Himari's same background and preparation, practicing in the same locality, would have taken in the same set of circumstances. This is the legal standard of care to which counselors are held. Himari's attorney would arrange for a different set of experts to testify that Himari did act reasonably in this particular situation. The judge or jury would have to decide which expert witnesses to believe.

After offering evidence through expert witness testimony that Himari breached her duty of care, Sheila's attorney would have to prove that Mark was injured as a result of this breach. It would not be difficult to prove that harm occurred, because Mark killed himself.

Next, Sheila's attorney would have to prove that there was a close causal connection between what Himari did, or failed to do, and Mark's suicide. Sheila's attorney would have to prove that Himari knew or should have known that her actions would result in Mark's death. The attorney would argue that Himari's failure to take action when Mark disclosed that he intended to attempt suicide was the proximate cause of Mark's death because she failed to take actions that would have prevented him from attempting suicide. On the opposing side, Himari's attorney would argue that a number of other intervening factors led to Mark's suicide and that a reasonable counselor would not and could not have predicted that Mark would take his life, given the circumstances in this situation. Further, Himari's attorney would argue that even if Himari did act negligently in this situation, Mark's suicide cannot be blamed on her failure to take action. Himari's attorney would argue that other factors *caused* his suicide—factors such as his father leaving his gun out so that Mark had access to it, no one being at home to supervise him after school, or his distress over his parents' contentious behavior during the divorce. The judge or jury would have to decide whether Himari's failure to act to prevent Mark's suicide was the proximate cause of his death, or whether other factors caused him to take his life.

If Sheila's attorney was successful in convincing the judge or jury that Himari was responsible for Mark's death, then the damage to Sheila, expressed in financial terms, would have to be determined. If a person is damaged with a financial loss, such as a loss in the stock market or because of investing in a deal that has no value, then it is easy to determine how much money the person needs in order to be *made whole again*. When a person has a physical injury, however, or loses a loved one through death, then it is very difficult to determine how much money it would take to compensate the person for the loss. In fact, these are losses that cannot be compensated with money, but the law requires financial compensation.

Damages in this situation would be determined by expert witness testimony on Mark's life expectancy, his earning potential, and the anticipated benefit of earnings that would go to his mother, Sheila. Because the loss of a life was involved in this situation, the damages could be hundreds of thousands or several million dollars. Judges or juries have wide discretion in setting the value in these kinds of cases.

If Sheila received a judgment in her favor, the school system's professional liability insurance carrier would most likely pay the judgment. Himari's professional liability carrier might be responsible for paying a portion of the judgment as well. Both the school's and Himari's insurance companies would pay the attorneys' fees and other costs of litigation, which could be as much as several hundred thousand dollars.

At this point, you probably are wondering how often lawsuits are based on client suicide and how often family members prevail and collect judgments. Such lawsuits are rare, and counselors are seldom determined to be responsible for the deaths of their clients who died by suicide.

Although client suicide is not the only possible basis for a malpractice lawsuit against counselors, it is undoubtedly one of the most painful types of suits for both plaintiffs and defendants. In the following section, we review some actual cases related to client suicide.

Actual Malpractice Cases

In 1960, a college counselor was sued for the wrongful death of a student who had died by suicide. The student had seen the counselor in a professional capacity (*Bogust v. Iverson*, 1960). The holding in that case was that the college counselor was not a therapist and therefore had no duty to detect or prevent a suicide of a client. Since 1960, however, the counseling profession has evolved substantially, and counselors in all settings are considered mental health professionals. A similar case today would probably have a different result.

A case that is similar to *Bogust v. Iverson* (1960) and to the hypothetical case of Himari was decided in Maryland in 1991. In *Eisel v. Board of Education of Montgomery County* (1991), Dorothy Jones and Deidre Morgan, school counselors at Sligo Middle School in Maryland (along with their school board, principal, and superintendent), were sued in a wrongful death action by Stephen Eisel, the father of Nicole Eisel. Nicole Eisel was a 13-year-old student at Sligo Middle School. Nicole and another 13-year-old girl consummated an apparent murder–suicide pact on November 8, 1988. The complaint filed in the lawsuit was summarized in the case report:

> The amended complaint avers that Nicole became involved in Satanism, causing her to have an "obsessive interest in death and self-destruction." During the week prior to the suicide, Nicole told several friends and fellow students that she intended to kill herself. Some of these friends reported Nicole's intentions to their school counselor, Morgan, who relayed the information to Nicole's school counselor, Jones. Morgan and Jones then questioned Nicole about the statements, but Nicole denied making them. Neither Morgan nor Jones notified Nicole's parents or the school administration about Nicole's alleged statements of intent. Information in the record suggests that the other party to the suicide pact shot Nicole before shooting herself. The murder–suicide took place on a school holiday in a public park at some distance from Sligo Middle School. (*Eisel v. Board of Education of Montgomery County*, 1991, pp. 449–450. Court of Appeals of Maryland)

Attorneys for the school board and its employees argued that school guidance counselors had no duty to recognize or prevent an adolescent's suicide. A motion for summary judgment in favor of the school board and its employees was granted, which means that the court, without even hearing the evidence in the case, found that the counselors were not responsible. Eisel appealed to the Maryland Court of Appeals. That court reversed the summary judgment and required that the case go back to the trial court so that a judge or jury could determine whether the counselors would be held responsible for Nicole Eisel's death.

In coming to this decision, the Maryland Court of Appeals held that "school counselors have a duty to use reasonable means to attempt to prevent a suicide when they are on notice of a child or adolescent student's suicidal intent" (*Eisel v. Board of Education of Montgomery County*, 1991, p. 456. Court of Appeals of Maryland). The decision noted that the counselors could have determined that Nicole was in danger of suicide even though she denied it when asked, that the school board had a policy requiring counselors to notify parents despite any confidentiality concerns, and that the school had a formal suicide prevention program in place. The decision stated, "The relationship of a school counselor and pupil is not devoid of therapeutic overtones" (p. 452). The appeals court did not decide that the two school counselors were responsible for Nicole Eisel's suicide, but it did decide that a judge or jury could determine that the facts were such that the counselors could be held responsible.

It is almost impossible to determine how many lawsuits of a particular nature are filed in court. Most lawsuits are settled without a judgment being rendered or are determined at the trial court level

and are not appealed. Only cases that are appealed are reported in case books. From the cases that are appealed, however, it appears that very few lawsuits are filed against counselors due to client suicide, and few of those filed result in judgments against the counselors. Nonetheless, counselors certainly hope to avoid experiencing the double trauma of having a client complete suicide (Stickl et al., 2021) and being named as a defendant in a lawsuit resulting from that suicide. However, if you practice competently, the chances are good that you will never have to respond to a malpractice lawsuit.

RESOLVING LEGAL AND ETHICAL CHALLENGES

In Chapter 1, ethical and legal decision making were discussed. Several ethical decision-making models were described, and common elements of those decision-making models were identified to help you recognize the key points to consider when you are faced with an ethical dilemma. Reasoning through a dilemma before taking action to resolve it can prevent later problems, such as being accused of improper or unethical behavior. This chapter section, by contrast, focuses on how to deal with suspicions or accusations of unethical or illegal conduct made against you when they do occur. Chapter 1 also presented a process for identifying legal issues when they arise and ways to request legal advice as an employee or as a counselor in private practice. It is one thing to understand rationally how to recognize legal issues and obtain legal advice, but it is quite another thing to feel the distress of dealing with ethical and legal issues when they actually arise.

Legal and Ethical Decision Making

This section of the chapter is aimed at helping you deal with situations that directly involve you in ethical and legal challenges that come before ethics committees, licensure or certification boards, and courts of law. After legal and ethical decision making are briefly reviewed, two kinds of situations are addressed: (a) what to do when you are accused of unethical or illegal behavior and (b) what to do if you suspect that a professional colleague is behaving unethically or illegally. Some guidelines are suggested for preventing ethical and legal problems and for maintaining your professional vitality.

PERSONAL LEGAL DECISION MAKING It is impossible for counselors to understand all aspects of the law. As a result, you must rely on the advice of attorneys from time to time as you practice. You must make every effort to fully explain a situation to your legal advisors and educate them, to the extent possible, regarding the counseling process and the counseling profession. You can then follow the legal advice you have been given without worrying about whether it is correct. Of course, you should raise questions if you believe you are being given incorrect advice, but you can rely on the advice you have been given. Lawyers are required to stand behind the advice they render. You can sue your lawyers if you are harmed as a result of relying on their incorrect advice. An attorney who has represented mental health professionals when they were sued (D. Gould, 1998) has stated that the best advice he can give to those who are involved in litigation and are trying to protect themselves from attacks is to listen to their lawyer and do what the lawyer says.

Most legal issues faced by counselors involve acting as witnesses in litigation concerning other people. Unless you are being sued for malpractice or have had a complaint of improper conduct filed against you, it is important to remember that you are not being accused of wrongdoing. Reporting what you know in legal proceedings can be a simple matter if you have been advised by your attorney. It is important to avoid getting emotionally involved in your clients' lawsuits or in legal proceedings that affect others. If you lose your objectivity and professional detachment, it is easy to get inappropriately involved in legal matters that do not affect you directly. See Chapter 6 on responding to subpoenas and Chapter 12 for advice on avoiding court appearances related to child custody disputes.

PERSONAL ETHICAL DECISION MAKING No doubt it has become evident to you, after reading the previous chapters, that ethical questions rarely have a single, clearly correct answer (Koocher & Keith-Spiegel, 2016). Codes of ethics can be helpful, and certainly they should be consulted. Remember, though, that ethics codes are written in general terms and cannot completely resolve most of the complex ethical dilemmas that arise in practice (Johnson et al., 2022; Williams, 2001). Often, counselors' ethical problems are existential dilemmas. The decisions they make when attempting to resolve an ethical issue become the right decisions only because they were taken. It is impossible to guarantee in advance that actions, no matter how carefully considered, will have the desired outcome.

Ethical decision making is a complex process, even in the abstract. See Chapter 1 for a discussion of the ethical decision-making process. It can be very difficult to make a decision when you are unsure about the ethics of your behavior, or you suspect that a colleague is behaving unethically. Of course, when you have an ethical question, you should consult with fellow counselors, just as counselors who have a legal question should consult with an attorney. As was noted in the previous section, when a counselor asks an attorney for advice, the attorney becomes responsible for the advice given. In a parallel fashion, sometimes a counselor with an ethical question will have an administrative supervisor to whom the responsibility for decisions can be passed. Often, though, this option will not be available. When you have an ethical question, you can—and should—consult with colleagues, but the consultants are not responsible for your decisions or actions. Although an attorney can help you defend your actions later before an ethics committee or a licensing board, if that becomes necessary, an attorney cannot advise you about an ethical dilemma.

Responding to Accusations of Unethical or Illegal Behavior

Neukrug et al. (2001) asked counselor licensure boards throughout the United States about ethical complaints that had been made against licensees. Of the 2,325 complaints recorded by the boards, a total of 1,307 were identified by type of complaint. Of the categorized complaints received by these boards, 24% were for inappropriate dual relationships, 17% were for incompetence, 8% were for practicing without a license or misrepresentation of qualifications, 7% were for having a sexual relationship with a client, 5% were for breach of confidentiality, 4% were for inappropriate fee assessments, 1% were for failure to obtain informed consent, and 1% were for failure to report abuse. In a study of ethical complaints against licensees in licensure boards from 2010 to 2014, Wilkinson et al. (2019) found that the most frequent types of ethical violations included failure to attain required continuing education, both sexual and nonsexual dual relationships, and misrepresentation of credentials.

A professional liability insurance company report indicated that the highest number of license protection claims from 2015 to 2019 were related to complaints of failure to maintain minimal professional standards, sexual misconduct, breach of confidentiality, and reporting to third parties (CNA & CNA & Healthcare Providers Service Organization [HPSO], 2019). This information demonstrates that counselors have complaints filed against them for a variety of reasons. As Keith-Spiegel (2014) has noted, ethics complaints sometimes get filed as a result of professionals losing their temper, disclosing personal information, entering into a bartering arrangement, and being involved in a vengeful act. Five times more mental health professionals have licensure board complaints filed against them than have been the target of malpractice lawsuits (Zur, 2011b).

Individuals react in various ways to accusations of unethical or illegal behavior (Warren & Douglas, 2012). Emotions such as fear, panic, and anger are common. It is natural to have a strong reaction, but you must keep your emotions under control so that you can handle the situation effectively. Chauvin and Remley (1996) recommend that you seek personal support by expressing your emotions regarding the accusation to clinical supervisors, family members, friends, or your personal counselor. However, you should avoid talking to them about the facts of the situation.

Despite the denial, embarrassment, or distress you may feel, if you are employed in an agency or institution, you should immediately report any threatened or completed accusation of professional misconduct to your immediate administrative supervisor and to your personal professional liability insurance carrier. If you fail to notify your insurance carrier, you might jeopardize the legal representation your insurance policy provides, such as paying off possible judgments. When you tell your supervisor about the accusation, you should ask the supervisor to keep the information confidential and not discuss it with anyone other than other administrative supervisors within the agency or the organization's attorney.

In reality, it is not unusual for counselors to be threatened with ethics complaints or malpractice lawsuits. Counselors engage in many activities that could result in ethics claims against them. They must report cases of suspected child abuse (Ricks et al., 2022), vulnerable adult abuse, or elder abuse; they counsel clients who are involved in litigation such as child custody disputes; and they often counsel individuals who are embroiled in legal conflicts with others. Sometimes counselors must take actions their clients do not wish them to take, such as informing family members, over a client's objection, if the client is at risk for suicide. As a result, individuals occasionally are angry at counselors and make threats. Diesen (2008) created a new term, *justice obsession syndrome*, to describe individuals who believe that the most important thing in life is to get justice in a personal situation where they firmly believe that an injustice has occurred. An obsession occurs, according to Diesen, when the importance of retribution is out of proportion to the circumstances. At some point in your counseling practice, you may have a client who feels wronged as a result of what has transpired in counseling sessions with you and becomes obsessed with seeking justice for the perceived wrong. As with other syndromes, the chances of having such a client are not great, but this type of behavior by clients has been known to occur.

Some counselors serve as child custody evaluators. There is always a disappointed—and often angry—parent when a child custody recommendation is made to a judge. Some parents who are upset file ethics complaints against child custody evaluators, accusing them of being biased and unfair.

A counselor who is dealing with an angry client should respond directly to the person making the threat, if that is possible. If you can calm the person down and listen to the concerns, you might be able to resolve the problem immediately or through additional conversations (Keith-Spiegel, 2014). You should be careful, though, not to admit to wrongdoing and not to say anything you would not want to say under oath or would not want repeated by others in a courtroom later. We advise caution on your part if a client you are counseling has a history of filing lawsuits against other people for any reason. Clients who sue other people may be more inclined to sue their counselors as well. Once a formal complaint or lawsuit has been filed, it is unwise to contact a client and try to talk them into withdrawing the complaint (O'Mahony, 2011).

INFORMAL COMPLAINTS The Introduction to Section I of the *Code of Ethics* (ACA, 2014) states, "Professional counselors behave in an ethical and legal manner. They are aware that client welfare and trust in the profession depend on a high level of professional conduct." This introduction also states, "Counselors incorporate ethical practice into their daily professional work and engage in ongoing professional development regarding current topics in ethical and legal issues in counseling."

Although we hope that you will always do your best to practice within the bounds of professional ethics, even the well-intended counselor occasionally can make mistakes or be accused of wrongdoing, even if no mistake has been made. A fellow counselor might directly observe you doing or saying something that raises cause for concern, or the concern might have come to your colleague's attention through the report of a client, fellow mental health professional, supervisee, or student. It is important to take the concern seriously, make yourself available to meet and discuss the concern, listen and respond genuinely and nondefensively, and take responsibility for your error if you have made one. If the concern can be resolved informally in this way, it is to everyone's advantage.

FORMAL COMPLAINTS Sometimes clients or other citizens file formal complaints against counselors with licensure boards, certification boards, professional associations, or other entities. When such complaints are filed, it is vital that counselors take them seriously and respond appropriately.

8-3 The Case of Lee

Lee has been licensed by her state board as a counselor for 4 years and is employed as a counselor in a community counseling center. Much to her dismay, Lee received a registered letter at work today informing her that a former client, David, has filed an ethics complaint against her with the licensure board, claiming that he was harmed because Lee is an incompetent counselor. David claims that Lee counseled him for 8 months for depression, during which time she sent him to a psychiatrist for antidepressant medication. David found out recently that he has a brain tumor, which his neurosurgeon has told him causes people to be depressed. David claims that if Lee were competent, she would have advised him to have a physical examination and that the brain tumor would have been discovered earlier. Lee immediately advised her supervisor at the counseling center and called and reported the complaint to her professional liability insurance carrier. After she emailed a copy of the complaint to her insurance carrier, she received a call from an agent telling her that the insurance company wanted Lee to see a local lawyer who would represent Lee. Lee has never heard of the lawyer.

- How should Lee respond to her insurance carrier?
- When Lee meets with her lawyer, how should she conduct herself?

Discussion: Lee should ask her insurance carrier how that lawyer was selected. Usually, lawyers are chosen who have expertise in defending mental health professionals or other professionals in ethics complaints before licensure boards. Insurance companies generally select the best lawyers in the area to represent the professionals they insure.

Lee should agree to meet with the lawyer who has been suggested by the insurance company and accept that lawyer to represent her, unless there is some compelling reason not to do so. If Lee does not like the suggested lawyer, she can ask the insurance carrier to hire another lawyer whom she prefers, and usually the insurance carrier will agree to do that if the alternative lawyer has expertise in defending suits against mental health professionals.

When Lee meets with her lawyer, she should bring copies of all relevant records with her to the meeting. She should also compile a chronological listing of her interactions with the complaining client that is factual and detailed and bring a copy of that document to give to the lawyer.

Lee should tell the lawyer everything she knows about the situation even if she is concerned that perhaps she used bad judgment or made some mistakes. The lawyer has a privileged relationship with Lee and cannot tell anyone else what Lee has disclosed. The lawyer needs to know all details (including negative ones) in order to craft an appropriate defense for her.

If you receive notice from a licensure or certification board or from a professional association ethics committee that you have been accused of unprofessional conduct, you need immediate legal counsel (Zur, 2011b). The professional liability insurance policy offered to members of ACA provides legal counsel to policyholders who have complaints filed against them. Generally, other professional liability insurance companies provide the same benefit to counselors that they insure. Counselors are advised to make sure the professional liability insurance policy they purchase includes legal representation in the

event a formal complaint is filed against them with their licensure board (Zur, 2011b). Some policies do not include that benefit and thus are not the best professional liability insurance options.

Licensure boards, professional association ethics committees, and counselor certification boards have adopted procedures for processing complaints against those who are licensed or certified by their boards or who are members of their association. The procedures used by ACA can be found on the ACA website (www.counseling.org). Each set of procedures is different; however, they have many common steps. The general process that boards and committees use in processing complaints is summarized in the following (Chauvin & Remley, 1996):

- Generally, investigations and the processing of complaints are confidential.
- First, a board will determine whether it has jurisdiction over the counselor against whom the complaint is being made. If the board does not have jurisdiction, it notifies the person making the complaint that it does not have jurisdiction, and the case is closed. If the individual is licensed or certified by the board (or is an applicant for a license or certification), then the board generally has jurisdiction.
- Once jurisdiction has been established, the board determines whether the counselor may have violated the board's code of ethics, if the allegations against the counselor are true. If the action (or inaction) of the counselor would not have constituted a violation, the case is closed. When the allegation, if true, might be a violation of the board's code of ethics, the board then begins an investigation of the complaint.
- Investigations might range from actions that are as expensive and complicated as sending investigators to talk to witnesses or the accused counselor, to actions as simple as asking written questions of the accuser or the accused.
- Often, if a violation is found, informal negotiations between the board and the accused counselor occur. A settlement of the matter may be reached on a voluntary basis that satisfies both parties.
- If a settlement is not offered by the board or is refused by the counselor, a hearing is generally held. Members of the board (or their designees) sit as a jury. A representative of the board prosecutes the case, and the accused counselor, or the counselor's representative, defends. Usually witnesses are called to testify, including the accuser. Accused counselors usually are allowed to be represented by legal counsel at these hearings, but an accused counselor may not be able to have a lawyer present in some jurisdictions.
- After the hearing, the board determines whether the counselor violated the code of ethics. If violations are found, the board may impose sanctions. Sanctions vary but could include a written reprimand only; probation; suspension; or revocation of a license, certification, or association membership.
- If the counselor who has been found in violation of the code of ethics is not satisfied with the result, the counselor can appeal. Usually, there are internal appeals in which the board reviews the case again. Once the internal appeals are exhausted, in most states the counselor can sue the board in a court of law if the counselor believes the treatment was unfair.

The process for responding to formal complaints against you filed with a state counselor licensure board is different from the way you will need to respond if a malpractice lawsuit is filed against you. If you receive official notice that a malpractice lawsuit has been filed against you, you must notify your employer and your professional liability insurance carrier immediately.

An emotional response to being accused of wrongdoing is natural. It may help to keep in mind that many formal complaints and lawsuits filed against mental health professionals are either dismissed or result in a finding in favor of the professional (Anderson & Swanson, 1994). In a study

FIGURE 8-2 Steps to take if you are accused of unethical behavior or are sued

- Maintain composure and respond at all times in a professional and unemotional manner.
- Notify your employer and professional liability insurance carrier immediately.
- Once you have a lawyer, follow the lawyer's advice precisely.
- Respond carefully to the charges. Remember that the members of the board or committee who will be deciding the outcome of the complaint do not know you and will base their decision only on the material they have received. Write your response as deliberately and dispassionately as you can, and address each of the specific charges that has been made. For instance, if you have been accused of misapplying a technique, you should submit case notes or other records that show how you deliberated and decided to use the technique, how you were trained to use it, when you consulted others about it, and any other steps you took to prevent harm to the client. Do not submit any responses until your attorney has reviewed them.
- Do not agree to speak to a licensure board investigator unless your attorney is present (Zur, 2011b).
- Even if you are surprised and hurt that a client or colleague has filed a complaint against you, do not attempt to contact the complainant directly. Doing so could be interpreted as an attempt to coerce or unduly influence the client or colleague (Crawford, 1994).
- Seek emotional support from colleagues, friends, or family members, but focus your interactions on dealing with your emotions. Ask anyone you speak with to keep your conversations confidential. Avoid discussing details of the case except with your attorney.
- Seek professional counseling if being accused results in distress that interferes with your personal or professional functioning.
- If you are guilty of having breached the code of ethics, seek assistance to help you prevent future breaches. This assistance could take the form of professional counseling, continuing education, supervision, or a combination of these resources.

of complaints against rehabilitation counselors over a 9-year period, Saunders et al. (2007) found that only 71 of the 113 complaints filed accusing rehabilitation counselors of unethical conduct were accepted for investigation. Actions or sanctions were taken against the counselors in only 36 of the 71 cases. It may also be helpful to know that severe sanctions against counselors are rarely imposed by the ACA Ethics Committee; for example, only two members were expelled from the association between 1996 and 2000 (Williams, 2001). In most formal complaint or lawsuit situations, the individuals being accused wish they had done or said some things differently. This is natural as well. To prevent making mistakes that might be detrimental to your case, Chauvin and Remley (1996) and Herlihy and Corey (2015a) have advised that you take the steps shown in Figure 8-2 if you are accused or sued.

WHEN YOU SUSPECT A COLLEAGUE IS ACTING UNETHICALLY OR ILLEGALLY

Unethical Behavior

The primary purpose of the *Code of Ethics* (ACA, 2014) is to guide our own behavior, not to judge the behavior of others. However, we cannot turn our heads and ignore unethical behavior on the part of others, particularly if such behavior is substantially harming others or has the potential of causing substantial harm. It is each counselor's responsibility to society and to the profession to address in an appropriate manner the behavior of other mental health professionals that appears to be unethical. Generally, mental health professionals have been reluctant to confront the unethical behavior of

a colleague (Lowman, 2006). The role of *policeperson* can be very uncomfortable and runs contrary to who we are as counselors and to our commitment to accepting and helping others rather than judging and punishing them. Nonetheless, we must recognize that unless we police our own profession, the courts will step in and do it for us.

The Introduction to Section I of the *Code of Ethics* (ACA, 2014) states, "Counselors strive to resolve ethical dilemmas with direct and open communication among all parties involved and seek consultation with colleagues and supervisors when necessary." This standard clearly communicates to counselors that they should consult when they are unsure whether another counselor is behaving in an unethical manner and should establish *direct and open communication* with the other counselor. When consultation and confrontation do not work, counselors are advised by the ACA *Code of Ethics* (2014), in serious cases, to take additional actions.

Standard I.2.a. of the *Code of Ethics* (ACA, 2014) states, "When counselors have reason to believe that another counselor is violating or has violated an ethical standard and substantial harm has not occurred, they attempt to first resolve the issue informally with the other counselor if feasible, provided such action does not violate confidentiality rights that may be involved." Two phrases in this standard require careful consideration. First, what constitutes a *reason to believe*? And second, what constitutes *resolution of the issue*?

In determining whether you have reason to believe that a colleague is acting unethically, we caution you against making a rush to judgment. You may hear rumors from colleagues that a fellow mental health professional engages in inappropriate behaviors such as sexually harassing clients or billing insurance companies for services that are not actually provided. Although you would be justified in feeling concerned, it would be unwise for you to take on the role of detective and investigate whether the rumors are true. Instead, urge the persons who are telling you the rumors to talk *to* the professional who is supposedly behaving inappropriately rather than talking *about* the individual. At other times, a client might tell you directly about unethical behavior on the part of a former counselor. In these instances, your role can be one of helping the client to decide whether to take action, if the client has been harmed by the former counselor's behavior. Only when you have direct knowledge of unethical behavior on the part of a fellow mental health professional should you feel obligated to take some type of action, but you need to be clear what your role is in such situations.

With respect to what constitutes resolving the issue, as we have noted, the *Code of Ethics* asks counselors to "attempt to first resolve the issue informally with the other counselor if feasible" (ACA, 2014, §I.2.b.). It appears that some confusion may exist regarding this ethical obligation. Nearly 30% of the ACA members surveyed by Neukrug and Milliken (2011) believed it was ethical to report a colleague's unethical conduct without first consulting with the colleague. Of course, you will be uncomfortable and anxious if you must confront a counselor about your perception that they may have violated an ethical standard. Such feelings are understandable; however, accusing a counselor of wrongdoing is a very serious step, and your anxiety about accusing someone of possible wrongdoing is not a justifiable reason for not attempting to resolve the issue directly with a counselor. It is entirely possible that speaking informally with the other counselor will resolve the problem and thus prevent further discomfort for both parties.

8-4 The Case of Ramona

Ramona is a counselor educator. At her university, the counseling program is housed within a large department that includes counseling, educational administration, and school psychology. One day Ramona is visiting an intern at a United Way–funded agency. She notices a flyer on the bulletin board that announces, "John Smith, PhD, will be forming a counseling group for men who are divorced." John is a colleague in her department at the university. He has a master's degree in counseling, but his PhD is in educational administration. She realizes that his advertising himself on the flyer as holding a PhD may be a violation of the *Code of Ethics* (ACA, 2014, §C.4.a. and §C.4.d.).

- Do you think that Ramona can ignore the situation? Or do you think she should do something about this possible ethics violation?
- If Ramona should act, what actions should she take?

Discussion: If Ramona were to decide to report John's behavior to their supervisor, to the state counselor licensure board, or to the ACA Ethics Committee, she would be creating problems—probably unnecessarily—for John and for their ongoing relationship as colleagues. Her first action should be to seek informal resolution by talking directly with John, pointing out that §C.4.a. and §C.4.d. in the *Code of Ethics* (ACA, 2014) are of concern. John may reply that he was unaware of these standards and had no idea that his advertisement might be a breach of the code. Whether or not John's advertisement is an ethics violation is open to question, as a PhD in educational administration could possibly be interpreted by some people as being in a field closely related to counseling. Ramona's goal should be to correct the problem, not to punish John. She can talk with him about what he can do to correct the situation. This might include taking down the flyers and printing new ones or being sure to clarify his credentials during the pregroup screening interview with potential group participants. If John were willing to take the steps agreed upon, Ramona probably would determine that the problem had been satisfactorily resolved.

What if the preceding scenario were different and John, rather than misrepresenting his PhD credential, had been drinking at a bar, encountered a former client, and had become flirtatious with the former client? What if the client had been uncomfortable seeing John in this situation and had left the bar? We believe that the total situation would dictate Ramona's actions. If Ramona believed this was an isolated incident, and that the client had not been substantially harmed as a result of the incident, then Ramona should confront John and hopefully obtain an agreement that he had made a mistake and a promise not to engage in such actions again. On the other hand, if Ramona believed that John had a history of sexual or romantic relationships with clients or former clients, or had a drinking problem that was interfering with his ability to counsel effectively, and if the client in this incident had been substantially harmed, then a confession and promise from John probably would not satisfy Ramona's obligation to the profession and to the public.

You will find that an informal resolution is not always possible when you have knowledge that another counselor may have engaged in unethical practices. The offending colleague may be unwilling to discuss your concern; circumstances may exist that preclude you from confronting your colleague; or you may not be able to raise your concern without violating the confidentiality of an involved party. If the privacy of an involved party is at risk, you should consult with colleagues who are uninvolved and who will not be able to ascertain the identity of the subject of the consultation.

FIGURE 8-3 Recommended necessary conditions for reporting a colleague for unethical behavior

- It is impossible to resolve the issue directly with the counselor because of the nature of the circumstances or because attempts at resolution have been unsuccessful. Being uncomfortable confronting the counselor or anxious about accusing someone of wrongdoing is not a justifiable reason for not attempting to resolve the issue directly with the counselor.
- You are certain and have direct knowledge that a serious ethics violation has occurred that is causing substantial harm or has caused substantial harm.
- Colleagues with whom you have consulted agree that a report must be made.
- You are willing to participate in a hearing and to testify against the counselor if a hearing is held.
- You are prepared to defend yourself if you have a counterclaim filed against you.

If someone has been substantially harmed or is being substantially harmed by the unethical behavior, and if your consultants agree that you must take action, you must go forward and report the behavior (Gricus & Wysiekierski, 2021; Smith, 2006). These circumstances will be rare. Standard I.2.b. of the *Code of Ethics* (ACA, 2014) states, "If an apparent violation has substantially harmed or is likely to substantially harm a person or organization and is not appropriate for informal resolution or is not resolved properly, counselors take further action depending on the situation. Such action may include referral to state or national committees on professional ethics, voluntary national certification bodies, state licensing boards, or appropriate institutional authorities. The confidentiality rights of clients should be considered in all actions. This standard does not apply when counselors have been retained to review the work of another counselor whose professional conduct is in question (e.g., consultation, expert testimony)." We suggest that counselors report suspected ethics violations of other counselors to licensure and certification boards, and to association ethics committees, only when the conditions in Figure 8-3 have been met.

The last two items on the list may come as a surprise to many counselors, but they are understandable when the ACA Ethics Committee's policies and procedures for dealing with complaints and the procedures of licensure and certification boards are taken into consideration. For example, the ACA Ethics Committee accepts only written complaints that are signed by complainants. The committee does not accept anonymous complaints. If you believe you must file a complaint and if the board does not provide a secure portal for submitting complaints, you should write a letter to the Ethics Committee outlining the nature of the complaint, sign it, and send it in an envelope marked *Confidential*. If the person against whom you are making the complaint is an ACA member or was a member when the alleged violation took place, the committee will send you a formal complaint that identifies all the ACA ethical standards that might have been violated, if the accusations are true. You will be asked to sign the complaint (after suggesting modifications, if needed) and a release-of-information form. The accused member then will receive copies of the formal complaint and any evidence or documents you have submitted to support your complaint. (For more complete information, see the ACA Ethics Committee's policies and procedures for processing complaints of ethical violations, which can be found on the ACA website at www.counseling.org.)

It is very distressing for counselors to have ethics charges filed against them by professional colleagues. Rarely does an accused counselor simply admit wrongdoing. Instead, the counselor may become defensive and sometimes may even make accusations of unprofessional behavior against the accuser. Usually, a long period of time passes while a complaint is being investigated and considered. The aftermath of the filing of an ethics complaint is stressful for both the accused and the accuser.

If you ever decide to file a complaint, you should precisely follow the instructions for filing the complaints. It is recommended that, before formally filing a complaint, you consult with an attorney and follow the attorney's advice. In many instances, depending on the circumstances and credentials of the counselor being accused, multiple complaints may be appropriate. Complaints may be filed at the same time with all of the following entities:

- All state licensure boards that have licensed the counselor
- All national certification boards that have certified the counselor
- Ethics committees that accept complaints of all professional counseling associations in which the counselor holds active membership
- The counselor's employer

Licensure boards, certification boards, and professional associations will inform individuals who inquire whether an individual comes under their jurisdiction and whether they accept ethics complaints against members. In the counseling profession, complaints usually are filed with and processed by ACA rather than with local or state chapters. When local chapters, state branches, regions, and divisions of ACA receive ethical complaints against members, they usually refer them to the national ACA Ethics Committee. On the other hand, the National Association of Social Workers and the American Psychological Association (APA) process ethics complaints against members on a local or state level. If an association processes complaints at the local level, information on the proper way to file complaints is available from the association's national office.

Unwarranted Complaints

Standard I.2.e. of the *Code of Ethics* (ACA, 2014) states, "Counselors do not initiate, participate in, or encourage the filing of ethics complaints that are retaliatory in nature or are made with reckless disregard or willful ignorance of facts that would disprove the allegation." This standard provides a foundation for finding counselors in violation of the ACA *Code of Ethics* if they purposefully file a formal ethics complaint against another counselor when they know the complaint is false or if they were not careful to gather and state facts accurately.

Inappropriate Discrimination Against Those Who Have Been Accused

Standard I.2.f. of the *Code of Ethics* (ACA, 2014) attempts to guard against counselors being discriminated against when they have had ethics complaints filed against them, even though the complaints have not yet been resolved. Even the most ethical counselors could have complaints filed against them. All it takes to file a complaint is submitting an accusation to the proper authorities. In addition, the ethical standard makes an effort to guard against counselors being discriminated against because they have filed complaints against other counselors. This standard states, "Counselors do not deny individuals employment, advancement, admission to academic or other programs, tenure, or promotion based solely on their having made or their being the subject of an ethics complaint. This does not preclude taking action based on the outcome of such proceedings or considering other appropriate information."

Despite this standard, if a complaint against a counselor is pending, employers certainly may conduct their own investigations into alleged wrongdoing and conclude that a counselor has been unethical, even if a complaint filed with a licensure board, certification board, or professional association still has not been resolved.

Illegal Behavior of Others

Individuals in U.S. society who know of illegal acts by others are not violating any laws if they choose not to report such activities. In the same manner, counselors who know of illegal acts of other counselors generally do not have to report those illegal acts. A citizen in the United States cannot be charged with a crime for refusing or failing to report the criminal act of another person. However, some illegal acts by counselors might also be unethical or might violate state licensure statutes or regulations. The previous section discusses how to proceed if you believe another counselor is acting unethically.

Counselors might become aware of the following illegal activities by other counselors:

- Lying to health care companies so that clients will get reimbursement for the counselors' services
- Purposefully not reporting income to the Internal Revenue Service
- Engaging in a sexual relationship with a minor
- Smoking in a building in which smoking is prohibited
- Speeding while driving a car

Obviously, you might decide to report some of these illegal activities even though you are not legally obligated to do so. Some activities might fall into a gray area, and you would not even think of reporting others.

Although counselors do not have an affirmative duty to report illegal activities of others, if they are questioned by police investigators or subpoenaed, they have to answer questions regarding the matter truthfully. Counselors cannot assist individuals who have broken the law if those individuals are attempting to avoid being questioned, arrested, or convicted of a crime they have committed.

Cases Are Often Complex

In this chapter, you have learned that you have an ethical duty to take appropriate action when you have good reason to believe that a fellow counselor is behaving unethically in a way that could cause substantial harm, and that sometimes you might choose to report an illegal activity. In Chapter 1, some ethical and legal decision-making strategies were offered. Throughout this text, case examples have been provided to help you hone your decision-making skills. You should keep in mind that most real-world ethical and legal problems are very complex and do not lend themselves to easy solutions.

8-5 The Case of Bill

Assume that you are a licensed professional counselor in private practice. In April of the past year, you were contacted by Bill, a counselor in the local high school. Bill was calling because he wanted to refer Chantelle, an 18-year-old student. You made an appointment with Chantelle and met with her in late April. You counseled her for two more sessions. In mid-May, she called to cancel her next session. While on the phone, she told you that she wanted you to understand why she was canceling and terminating the counseling relationship. She also wanted the reassurance that you would not talk with her school counselor.

In response, you reviewed with her the limits of confidentiality and assured her that you would not talk with Bill but said that if she told you of abuse or threatened harm to herself or others, you would have to report it to authorities. Chantelle then disclosed that she had been having romantic feelings for Bill for nearly a year and that she was upset and confused about the relationship.

You stressed to Chantelle that this would not be a good time to discontinue counseling. Chantelle promised to think things over and call you back.

Chantelle returned for one more session in June, soon after she had graduated from high school. She told you that she had decided that she and Bill should just be friends. She also disclosed that although she had not had sexual intimacies with Bill, he had talked of being attracted to her. Chantelle recalled that she had felt confused when Bill first flirted with her and now felt that her trust had been betrayed. When you asked her for permission to talk with Bill, she adamantly refused. You encouraged her to talk to the school officials, but, again, she was adamant in refusing, stating that she didn't want to get Bill into trouble. She added that she had never told anyone, including her parents, about what had happened and that she didn't want anyone besides you to know about it.

You know Bill personally, although the two of you are not close friends. You see him at meetings of professional associations and local continuing education workshops. You believe that Chantelle is telling the truth. At the same time, Bill has always impressed you as a competent professional.

- What are your professional and ethical responsibilities to Chantelle?
- What are your professional and ethical responsibilities to Bill?

Discussion: *Regarding your responsibilities to Chantelle:*

- You should try to get more information from Chantelle regarding the exact behaviors of Bill in an effort to determine whether Bill had behaved inappropriately or whether Chantelle may have misinterpreted his attention.
- You have an obligation to respect Chantelle's privacy, unless you determine that she may have been subjected to inappropriate romantic overtures from Bill.
- If you have reason to believe that Bill acted inappropriately, you would have an obligation to take some type of action to ensure that he would be held accountable.

Regarding your responsibilities to Bill:

- You should assume that Bill has done nothing wrong until you determine in your mind that he may be guilty of inappropriate behavior with Chantelle.
- If you decide that Bill may have been inappropriate, you should talk to him directly about the situation and hear his side of the story.
- After talking to Bill, if you believe Chantelle had been substantially harmed, or that Bill may substantially harm other students, then you should take actions to ensure that Bill is held accountable for his behavior.

Guidelines for Preventing Problems

No matter how carefully you practice or how conscientious you are as a professional, it is always possible that someone will accuse you of unethical or illegal behavior. Guidelines that have been suggested by Chauvin and Remley (1996) for preventing problem situations follow:

- Restrict your practice of counseling to your areas of competence.
- Do not sue clients for unpaid fees. Those who are sued by you may, in turn, complain about your competency. If you don't allow clients to run up bills with you, collecting fees from them will never become an issue.

- Utilize a thorough and complete client disclosure statement in your practice (Wheeler & Bertram, 2012).
- Never guarantee or imply a guarantee of outcomes.
- Establish and maintain firm professional boundaries between you and your clients.
- Always use supervision for your practice, even if it is peer supervision.

We suggest and briefly discuss a few additional guidelines:

- Stay connected with your fellow mental health professionals. Attend meetings of professional organizations and continuing education workshops. Join a peer consultation group that meets regularly. Isolation can create blind spots (Mascari & Webber, 2006), and counselors who are isolated from their colleagues are more prone to burnout and to inappropriately trying to meet their own needs through their relationships with clients.
- Monitor your self-disclosures made in counseling relationships. Check to make sure that you are self-disclosing for the benefit of your clients rather than to have your own needs met. This is one important way to prevent a subtle slide into blurred boundaries and inappropriate dual relationships.
- Document carefully any circumstances and actions taken in difficult or dangerous client situations (such as abuse or threats to harm self or others; Wheeler & Bertram, 2019).
- Keep current with developments in law and ethics. Ethical standards for the profession are not static, and in every state, laws and rules related to the practice of counseling are constantly changing (Hegarty, 2012). It is vital that you keep abreast of changes in requirements for practice and advances in knowledge in the field.
- To reiterate advice we offer many times throughout this text, when in doubt, *consult* (Melonas, 2011). As we note, decisions made in isolation are rarely as sound as those made in consultation with others. Choose your consultants wisely—fellow mental health professionals for ethical questions and attorneys for legal questions—and follow their advice.
- Continually monitor your own effectiveness as a counselor. You have an ethical duty (ACA, 2014, §C.2.e.) as well as a moral responsibility to do so. Recall the discussion of virtue ethics presented in Chapter 1. Virtuous counselors have such an abiding commitment to the ethical values of the profession that they hold themselves accountable even when others do not.

Be sure to take care of your own mental and emotional health. In our view, it is often the most conscientious counselors who fall prey to mental and emotional exhaustion. It may help to keep in mind that sound practice is not about achieving perfection; rather, it is about taking responsibility for one's actions and keeping client welfare foremost (Welfel, 2016). At the same time, we believe that there is an ethic of self-care (Brown, 2018; Wagner et al., 2020). As Corey et al. (2024) have stated, a key to retaining your vitality as a person and a professional is to realize that you do not have an unlimited capacity to give without replenishing yourself. Attending to your own wellness is fundamental to your ability to assist your clients in achieving their emotional health and wellness.

Counselors must take care of themselves legally as well. Professionals who are sued or accused of wrongdoing are not able to function to their maximum potential. We owe it to our clients to protect ourselves legally as we practice, to seek legal advice when it is needed, and to maintain professional liability insurance. By protecting ourselves legally, we are able to help others as professional counselors.

Summary and Key Points

This chapter addresses unfortunate topics related to clients being dissatisfied with the services of counselors. Counselor malpractice is reviewed. In addition, the process whereby counselors are accused of unethical practice is discussed.

Counselors are seldom sued for malpractice, but it is possible. In this chapter, the legal principles of malpractice are reviewed and explained. If counselors do get sued, the complaints often are sexual misconduct or failing to prevent the suicide of a client. Counselors could also be sued for malpractice if they fail to prevent harm if a client is at risk for violence.

- COVID-19 and the digital revolution in the profession of counseling have had an impact on how counselors deal with clients who might be at risk of suicide or violence.
- Counselor malpractice is defined as the failure of counselors to render professional services expected in a particular community by a reasonable member of the counseling profession with the result that a recipient of services is harmed. Malpractice can include intentional or unintentional wrongdoing.
- Malpractice lawsuits against counselors are difficult for a client to win because clients must show that counselors were guilty of wrongdoing and that the wrongdoing caused some actual harm to the client.
- Knowing how to proceed after determining that a client may be at risk for suicide or violence is essential for counselors who practice in all settings.
- It is important for counselors to have a good understanding of how to assess clients for nonsuicidal self-injury, bullying, and risk for suicide.
- Counselors in all states except Texas have a duty to warn known intended victims of their clients. However, state laws vary widely, so it is essential to know the law in the state in which you practice.

Counselors must be prepared to resolve different legal and ethical challenges. Understanding how to proceed when a legal question arises is different from resolving ethical or counseling practice issues.

- It is very important to consult with attorneys when legal questions arise in your counseling practice.
- On the other hand, other counselors, not lawyers, should be consulted when making difficult ethical or professional decisions.

Counselors sometimes have complaints filed against them with their state licensure board, a certification agency, a professional association, or their employer. Understanding how to deal with such complaints is important. Dealing with suspected unethical conduct by colleagues is an important situation to navigate to prevent having counter complaints filed against you.

- If you are accused of unethical practice, you should seek legal advice, avoid talking with others about the accusation, and respond in a professional manner to licensure boards or association ethics committees.
- The first step in addressing what appears to be unethical behavior by a colleague is to address the situation directly and to try to resolve the problem through that process. When that fails or in situations where direct interaction with the colleague is impossible and substantial harm has occurred or is occurring, you should consult with other counselors and, if necessary, report the situation to employers, licensure boards, or certification entities.
- You are not obligated by law to report illegal activities of other counselors, but you might have an ethical obligation if you believe substantial harm is occurring because of the other counselor's behavior.